

#1 · Care Management

What is case management?

Answer

Case management is a collaborative process of assessment, planning, facilitation, care coordination, evaluation, and advocacy for options and services to meet an individual's health needs. The goal is to promote quality, cost-effective outcomes while supporting the patient's self-management and autonomy.

#2 · Care Management

What are the six steps of the case management process?

Answer

The six steps are: screening and identification, assessment, care planning, implementation, monitoring, and transition or closure. Each step builds on the previous one to ensure the patient's needs are identified and addressed systematically.

#3 · Care Management

What is the purpose of the initial case management assessment?

Answer

The initial assessment gathers a comprehensive picture of the patient's clinical status, functional ability, psychosocial situation, support system, insurance and financial resources, health literacy, and goals. This full picture is required before a meaningful care plan can be developed.

#4 · Care Management

What does a comprehensive case management care plan include?

Answer

A care plan includes identified problems and needs, measurable goals with target dates, specific interventions, the responsible party for each intervention, and a timeline for monitoring and evaluation. It is developed collaboratively with the patient and updated as the situation changes.

#5 · Care Management

What is the difference between case management and disease management?

Answer

Case management is highly individualized and addresses complex patients across multiple conditions and life domains. Disease management is a population-level program using standardized protocols for patients with a specific chronic condition, typically conducted telephonically at lower intensity.

#6 · Care Management

What is risk stratification?

Answer

Risk stratification divides a patient population into groups based on likelihood of adverse outcomes or high utilization, so that case management resources can be matched to the level of need. High-risk patients receive intensive case management; low-risk patients receive lighter monitoring.

#7 · Care Management

What tools are used for risk stratification?

Answer

Common tools include predictive modeling software using claims data, clinical registries, and validated risk scores such as the Charlson Comorbidity Index. Multiple factors are considered including diagnosis burden, prior utilization, functional status, and social determinants.

#8 · Care Management

What is care coordination?

Answer

Care coordination is the deliberate organization of patient care activities and sharing of information among all participants involved in a patient's care to achieve safer and more effective care. It reduces fragmentation, prevents duplication, and ensures the right care is delivered at the right time.

#9 · Care Management

What is transitional care?

Answer

Transitional care refers to the coordination and continuity of care as patients move between care settings, such as from hospital to home or hospital to skilled nursing facility. Poor transitions are a leading cause of preventable readmissions and medication errors.

#10 · Care Management

What are the key elements of safe hospital discharge planning?

Answer

Safe discharge planning includes early initiation on admission, medication reconciliation, patient and caregiver education, confirmed follow-up appointments, and community services arranged before the patient leaves. The patient must understand the plan and be able to implement it.

#11 · Care Management

What is the 30-day readmission rate and why does it matter?

Answer

The 30-day readmission rate measures how many patients return to the hospital within 30 days of discharge. CMS uses this metric to assess hospital quality and penalizes hospitals with excessive readmissions for certain conditions, creating a financial incentive for effective discharge planning.

#12 · Care Management

What is patient activation?

Answer

Patient activation refers to the knowledge, skills, and confidence a person has to manage their own health and healthcare. Higher patient activation is associated with better self-management, lower utilization, and better health outcomes. Case management interventions can increase activation over time.

#13 · Care Management

What is self-management support?

Answer

Self-management support helps patients develop the skills and confidence to manage their chronic conditions day-to-day. It includes goal-setting, problem-solving, action planning, and building health literacy so the patient can make informed decisions between clinical encounters.

#14 · Care Management

What is the purpose of monitoring in case management?

Answer

Monitoring tracks the patient's progress toward care plan goals, identifies new barriers or problems, evaluates whether interventions are working, and prompts adjustments when needed. It is an ongoing, active process rather than a one-time check-in.

#15 · Care Management

What is an interdisciplinary care team?

Answer

An interdisciplinary care team includes professionals from multiple disciplines who work together toward shared patient goals. Members may include physicians, nurses, social workers, pharmacists, physical therapists, and case managers, each contributing their specific expertise.

#16 · Care Management

What is the case manager's role in an interdisciplinary team?

Answer

The case manager facilitates communication among team members, coordinates the care plan, advocates for the patient's perspective, and ensures the plan is implemented and monitored. They are often the hub of communication connecting all team members and the patient.

#17 · Care Management

What is a care conference?

Answer

A care conference is a structured meeting involving the patient, family, and members of the care team to discuss the patient's status, goals, and plan. It is especially useful when there is disagreement about the plan or when a complex transition is being planned.

#18 · Care Management

What is population health management?

Answer

Population health management proactively identifies and manages health outcomes for a defined population, using data analytics to stratify risk and target interventions. It includes healthy people, at-risk individuals, and those with established disease rather than waiting for patients to present with problems.

#19 · Care Management

What is an accountable care organization (ACO)?

Answer

An ACO is a group of providers and hospitals that voluntarily coordinate care for a defined population and take shared accountability for the total cost and quality of care. Financial incentives reward quality and efficiency rather than volume of services.

#20 · Care Management

What is a patient-centered medical home (PCMH)?

Answer

A PCMH is a primary care model providing comprehensive, coordinated, accessible, and quality-focused care through a team-based approach led by a physician or advanced practice provider. NCQA provides PCMH recognition, and the model emphasizes care coordination and proactive patient management.

#21 · Care Management

What is value-based care?

Answer

Value-based care ties reimbursement to quality and outcomes rather than volume of services. It creates financial incentives for healthcare organizations to prevent complications, reduce avoidable utilization, and invest in care coordination and prevention.

#22 · Care Management

What is telephonic case management?

Answer

Telephonic case management uses phone calls to assess, educate, coordinate, and monitor patients without in-person visits. It is scalable and can reach patients across geographic areas, though it cannot substitute for in-person assessment when physical observation is needed.

#23 · Care Management

What are social determinants of health?

Answer

Social determinants of health are the conditions in which people are born, grow, live, work, and age that affect health outcomes. They include economic stability, education, social support, neighborhood environment, and access to healthcare. Addressing them is a core case management function.

#24 · Care Management

How does food insecurity affect chronic disease management?

Answer

Food insecurity limits access to fresh, nutritious food needed for disease management, making dietary adherence difficult. It is associated with worse blood sugar control in diabetes, higher rates of hypertension, and increased hospitalizations. Case managers screen for food insecurity and connect patients to food resources.

#25 · Care Management

What is the role of community resources in case management?

Answer

Community resources such as food banks, transportation programs, housing assistance, senior services, and peer support groups are part of the care continuum. Connecting patients to these resources addresses social determinants that affect health outcomes and is a core case management function.

#26 · Care Management

What is health literacy?

Answer

Health literacy is the degree to which individuals can obtain, process, and understand basic health information and services needed to make appropriate health decisions. Limited health literacy is associated with worse medication adherence, more hospitalizations, and worse chronic disease control.

#27 · Care Management

What is the teach-back method?

Answer

Teach-back asks patients to explain information back in their own words to confirm understanding rather than just asking whether they understood. It is the most effective technique for identifying comprehension gaps and is a best practice for all patient education in case management.

#28 · Care Management

What is integrated behavioral health care?

Answer

Integrated behavioral health care places mental health and substance use services within primary care settings, removing the separation between physical and behavioral health. The Collaborative Care Model is one evidence-based approach using a care manager and psychiatric consultant in the primary care team.

#29 · Care Management

What is care management for high-risk patients?

Answer

High-risk patients with multiple chronic conditions, frequent hospitalizations, and complex social needs receive intensive case management with frequent contact, comprehensive care planning, and coordination across multiple providers and services. The goal is to stabilize the patient and prevent costly acute events.

#30 · Care Management

What is the CMSA Standards of Practice for Case Management?

Answer

The CMSA Standards define professional expectations for case management activities including assessment, planning, implementation, coordination, monitoring, and evaluation. They are developed by the Case Management Society of America and serve as a professional reference across all practice settings.

#31 · Care Management

What is caseload management?

Answer

Caseload management involves prioritizing patients based on acuity and complexity so that the most vulnerable patients receive the most intensive attention. It requires clinical judgment about contact frequency, escalation thresholds, and when to transition or close cases.

#32 · Care Management

What is the purpose of case closure?

Answer

Case closure occurs when case management goals are met, the patient no longer needs intensive support, or the patient declines services. Proper closure includes documentation, patient education about community resources, and a final assessment to ensure a safety net is in place.

#33 · Care Management

What is the difference between screening and assessment in case management?

Answer

Screening identifies patients who may need case management using brief tools or criteria such as high utilization or specific diagnoses. Assessment is a comprehensive, individualized evaluation of the patient's clinical, functional, psychosocial, and social needs that follows screening.

#34 · Care Management

What is motivational interviewing?

Answer

Motivational interviewing is a collaborative, person-centered counseling approach that helps patients explore and resolve ambivalence about change by drawing out their own values and motivations. It uses OARS: Open questions, Affirmations, Reflective listening, and Summaries.

#35 · Care Management

What is the Transtheoretical Model (Stages of Change)?

Answer

The Transtheoretical Model describes five stages: precontemplation, contemplation, preparation, action, and maintenance. Case managers match their interventions to the patient's current stage rather than prescribing the same approach to all patients regardless of readiness.

#36 · Care Management

What is change talk in motivational interviewing?

Answer

Change talk is patient language that reflects a desire, ability, reason, or need to change. The case manager reflects and reinforces change talk statements because they are associated with actual behavior change. Sustain talk, which favors the status quo, is acknowledged but not reinforced.

#37 · Care Management

What is action planning?

Answer

Action planning involves collaboratively developing specific, achievable, patient-chosen steps toward a health goal. It follows assessment of the patient's readiness, barriers, and resources, ensuring the plan is realistic and driven by the patient's own priorities rather than the case manager's agenda.

#38 · Care Management

What are SMART goals?

Answer

SMART goals are Specific, Measurable, Achievable, Relevant, and Time-bound. This structure makes goals concrete, trackable, and aligned with the patient's own capacity and priorities, making success more likely and making it easier to evaluate whether the plan is working.

#39 · Care Management

What is shared decision-making?

Answer

Shared decision-making is a collaborative process where clinicians and patients work together to make healthcare decisions that integrate the best available evidence with the patient's own values, preferences, and life circumstances. The patient is a co-decision-maker, not a passive recipient.

#40 · Care Management

What is the Health Belief Model?

Answer

The Health Belief Model proposes that people are more likely to take a health action when they perceive the condition as serious and themselves as susceptible, and when they believe the benefits of action outweigh the barriers. Case managers use it to understand why patients may not act on health recommendations.

#41 · Care Management

What is the Social Ecological Model?

Answer

The Social Ecological Model recognizes that health behavior is shaped by multiple interacting levels: individual, interpersonal, community, organizational, and policy. It explains why individual education alone may fail when interpersonal norms, community access, or policy barriers are driving behavior.

#42 · Care Management

What is self-efficacy?

Answer

Self-efficacy is a person's belief in their ability to successfully perform a specific behavior, a concept from Social Cognitive Theory developed by Albert Bandura. It is one of the strongest predictors of health behavior change and sustained self-management in chronic disease.

#43 · Care Management

What is harm reduction?

Answer

Harm reduction accepts that not all patients will stop harmful behaviors immediately and focuses on reducing the associated harms rather than requiring abstinence. Examples include providing clean needles, naloxone distribution, and supervised consumption sites. It is evidence-based and reduces HIV transmission and overdose deaths.

#44 · Care Management

What is empowerment in case management?

Answer

Empowerment means helping patients develop the knowledge, skills, and confidence to manage their own health rather than creating dependency on the case manager. It is a core goal of chronic disease management and is associated with better long-term outcomes than provider-directed care.

#45 · Care Management

What is strength-based case management?

Answer

Strength-based case management focuses on identifying and building on what is already working in a patient's life: coping skills, relationships, values, and previous successful change efforts. It addresses problems within a framework that builds on existing capacities rather than focusing primarily on deficits.

#46 · Care Management

What is patient-centered care?

Answer

Patient-centered care places the patient's values, preferences, and life circumstances at the center of care planning. The patient is recognized as the expert on their own life and an equal partner in decisions about their health, with clinical expertise integrated into rather than imposed on the plan.

#47 · Care Management

What is the role of advance care planning in case management?

Answer

Advance care planning helps patients clarify and document their healthcare values and wishes before a health crisis. Case managers facilitate conversations about goals of care, advance directives, and surrogate decision-makers, ensuring the patient's wishes are accessible across care settings.

#48 · Care Management

What is palliative care?

Answer

Palliative care is appropriate at any stage of a serious illness and focuses on improving quality of life by addressing pain, symptoms, and psychosocial needs. It can be provided alongside curative treatment and does not require a prognosis of six months or less.

#49 · Care Management

How does palliative care differ from hospice?

Answer

Palliative care is available at any illness stage and alongside curative treatment. Hospice is a specific palliative care program for patients with a prognosis of six months or less who have chosen comfort-focused care and agreed to forgo curative treatment. All hospice is palliative but not all palliative care is hospice.

#50 · Care Management

What is care management for patients with multiple chronic conditions?

Answer

Patients with multiple chronic conditions require care management that addresses all conditions together rather than treating them as separate problems. The case manager coordinates across multiple providers, identifies interactions between conditions and medications, and develops a unified care plan with clear priorities.

#51 · Care Management

What is the biopsychosocial model?

Answer

The biopsychosocial model recognizes that biological, psychological, and social factors all interact to affect health outcomes. It is the foundational framework for comprehensive case management assessment, which must address all three dimensions rather than focusing only on the medical diagnosis.

#52 · Care Management

What is trauma-informed care?

Answer

Trauma-informed care recognizes that a history of trauma affects how patients engage with healthcare. It creates systems and interactions that promote safety, trust, choice, collaboration, and empowerment rather than triggering re-traumatization. It applies to all patients, not only those in behavioral health settings.

#53 · Care Management

What is cultural humility?

Answer

Cultural humility is an ongoing process of self-reflection and learning that involves acknowledging the limits of one's own cultural knowledge and remaining open to learning from each patient. It recognizes power imbalances in the clinical relationship and treats the patient as the expert on their own cultural experience.

#54 · Care Management

What is the role of a professional interpreter in case management?

Answer

A professional medical interpreter ensures accurate, neutral communication between the case manager and a patient with limited English proficiency. Family members should not substitute for professional interpreters because they may filter information, lack medical vocabulary, or have their own agenda in the conversation.

#55 · Care Management

What is medication reconciliation?

Answer

Medication reconciliation creates an accurate, complete list of all medications a patient is taking and compares it to orders at admission, transfer, and discharge to identify discrepancies. It includes prescription, over-the-counter, and herbal medications and is required at every care transition to prevent medication errors.

#56 · Care Management

What is continuity of care?

Answer

Continuity of care ensures that information flows with the patient across providers and settings, so the care team always has the information needed to provide safe, appropriate, and consistent care. The case manager is a key facilitator of continuity, especially during care transitions.

#57 · Care Management

What is the purpose of a warm handoff?

Answer

A warm handoff is a direct, real-time introduction of a patient from one provider to another rather than simply providing a referral. It increases the likelihood the patient will follow through with the next provider and ensures the receiving provider has the context needed to continue care effectively.

#58 · Care Management

What is the role of the case manager in managing caregiver stress?

Answer

The case manager assesses caregiver burden, validates the caregiver's challenges, and connects caregivers with respite care, support groups, and community resources. Caregiver burnout directly threatens the sustainability of the patient's home-based care arrangement, making caregiver support a clinical priority.

#59 · Care Management

What is a goals-of-care conversation?

Answer

A goals-of-care conversation explores what matters most to the patient, what they understand about their illness, what they fear, and what they hope for, resulting in a care plan aligned with their values and priorities. It precedes specific decisions about treatment and advance directives.

#60 · Care Management

What is case management in the context of behavioral health?

Answer

Behavioral health case management coordinates mental health and substance use services, connects patients to community supports, monitors medication adherence, and addresses social determinants that affect recovery. Integration of behavioral and physical health case management produces better outcomes than siloed management.

#61 · Care Management

What is peer support in case management?

Answer

Peer support specialists are people with lived experience of a health or mental health challenge who are trained to provide support, share recovery strategies, and reduce isolation for patients facing similar challenges. They are a recognized evidence-based component of behavioral health and chronic disease programs.

#62 · Care Management

What is interdisciplinary versus multidisciplinary care?

Answer

Multidisciplinary care involves professionals from different disciplines working in parallel with separate plans. Interdisciplinary care involves shared goals, active collaboration, and an integrated care plan. Interdisciplinary teams produce better coordination and patient outcomes than multidisciplinary parallel work.

#63 · Care Management

What is care management for the elderly population?

Answer

Geriatric case management addresses the unique needs of older adults including functional decline, polypharmacy, cognitive impairment, fall risk, caregiver strain, and social isolation. Assessment tools include the Katz Index, IADL assessments, and fall risk scales. The goal is to maximize independence in the least restrictive setting.

#64 · Care Management

What is a home safety assessment?

Answer

A home safety assessment evaluates the patient's living environment for hazards that could cause falls or injury, accessibility barriers, medication storage safety, and adequacy of support for the patient's functional needs. It is typically performed by a nurse or occupational therapist and drives discharge planning and home modification recommendations.

#65 · Care Management

What is a post-discharge follow-up call?

Answer

A post-discharge follow-up call contacts the patient within 24-72 hours of hospital discharge to confirm the patient arrived home safely, understands the discharge plan, has filled prescriptions, and has a follow-up appointment scheduled. It identifies problems early before they lead to readmission.

#66 · Care Management

What are the key functions of case management in utilization management?

Answer

In utilization management, the case manager reviews clinical information to determine whether services meet medical necessity criteria, coordinates prior authorizations, manages concurrent review for inpatient stays, and facilitates peer-to-peer review when denials appear clinically inappropriate.

#67 · Care Management

What is concurrent review?

Answer

Concurrent review is ongoing evaluation of an inpatient hospital stay to determine whether the patient continues to meet medical necessity criteria for the level of care. The case manager collects clinical information and communicates with the payer to support continued stay authorization.

#68 · Care Management

What is the role of the case manager in appeals?

Answer

When a payer denies a service the case manager believes is clinically necessary, the case manager gathers supporting documentation, initiates an appeal, and requests peer-to-peer review between the treating physician and the plan's medical director. Advocacy obligations apply regardless of employment setting.

#69 · Care Management

What is a peer-to-peer review?

Answer

A peer-to-peer review is a conversation between the treating physician and the payer's medical director to discuss the clinical rationale for a service that has been denied. It often results in reversal of denials when the treating physician can present additional clinical evidence directly.

#70 · Care Management

What is prior authorization?

Answer

Prior authorization is a payer requirement that certain services be approved before they are provided. The case manager submits clinical documentation to demonstrate the service meets coverage criteria. Failure to obtain required authorization can result in claim denial even when the service was clinically appropriate.

#71 · Care Management

What is step therapy?

Answer

Step therapy (also called fail-first) requires that patients try less expensive or lower-risk medications before a higher-cost medication is approved. Exceptions may be requested when there is a clinical reason to bypass the stepwise requirement, such as prior treatment failure or contraindication.

#72 · Care Management

What is a Level of Care determination?

Answer

A Level of Care determination matches the patient's clinical needs to the appropriate setting, such as acute inpatient, observation, SNF, IRF, home health, or outpatient. The case manager uses standardized criteria and clinical judgment to ensure patients receive care in the least restrictive appropriate setting.

#73 · Care Management

What is site-of-care management?

Answer

Site-of-care management redirects services from higher-cost settings (hospital outpatient) to lower-cost clinically equivalent settings (ambulatory surgery center) when the plan requires or incentivizes this. The case manager communicates the requirement to the patient and provider and assists with arranging the preferred site.

#74 · Care Management

What is a Notice of Non-Coverage (HINN)?

Answer

A Hospital-Issued Notice of Non-Coverage (HINN) informs a Medicare patient that their hospital stay is no longer covered because they no longer meet inpatient medical necessity criteria. It triggers the patient's right to request a review by the Beneficiary and Family Centered Care Quality Improvement Organization.

#75 · Care Management

What is the role of case management in discharge to SNF?

Answer

The case manager verifies Medicare SNF eligibility (three-day qualifying inpatient stay), assesses the patient's therapy and nursing needs, identifies an appropriate SNF with available beds, communicates the plan to the patient and family, and arranges the transfer with complete clinical documentation.

#76 · Care Management

What is the Long-Term Care continuum?

Answer

The long-term care continuum includes home and community-based services, adult day programs, assisted living facilities, memory care units, and nursing facilities. The case manager matches the level of support to the patient's functional needs and preferences, aiming for the least restrictive appropriate setting.

#77 · Care Management

What is a Medicaid HCBS waiver?

Answer

Home and Community-Based Services (HCBS) waivers allow states to provide personal care assistance, home-delivered meals, and other support services that help individuals with disabilities remain in the community. These are optional Medicaid benefits with limited enrollment slots that often have waiting lists.

#78 · Care Management

What is care management for patients with substance use disorders?

Answer

Substance use disorder case management coordinates medical detoxification, residential or outpatient treatment, medication-assisted treatment, peer support, and housing. It applies motivational interviewing and harm reduction principles, recognizing that recovery is a non-linear process with potential relapse.

#79 · Care Management

What is Medication-Assisted Treatment (MAT)?

Answer

MAT uses FDA-approved medications including buprenorphine, methadone, and naltrexone combined with counseling to treat opioid use disorder. It is the gold-standard evidence-based treatment for opioid use disorder and significantly reduces overdose deaths, illicit drug use, and criminal activity.

#80 · Care Management

What is the role of case management in cancer care?

Answer

Cancer case management coordinates diagnosis, treatment, supportive care, and survivorship services while addressing the psychosocial impact of a life-altering diagnosis. Fear of recurrence, treatment side effects, financial toxicity, and social support needs are central case management concerns in oncology.

#81 · Care Management

What is financial toxicity in cancer?

Answer

Financial toxicity refers to the financial burden and resulting distress caused by cancer treatment costs, which can lead patients to skip doses, delay care, or exhaust savings. Case managers screen for financial toxicity and connect patients with pharmaceutical assistance programs, social work, and community resources.

#82 · Care Management

What is the role of case management in heart failure?

Answer

Heart failure case management focuses on medication adherence, daily weight monitoring, sodium and fluid restriction, early recognition of decompensation warning signs, and prompt access to the care team. Evidence-based heart failure case management programs significantly reduce 30-day readmissions.

#83 · Care Management

What is care management for patients with diabetes?

Answer

Diabetes case management addresses medication adherence, blood glucose monitoring, nutrition, physical activity, foot care, and preventive screenings. It uses the chronic care model, engages the patient as the primary self-manager, and coordinates across primary care, endocrinology, ophthalmology, and podiatry.

#84 · Care Management

What is the Chronic Care Model?

Answer

The Chronic Care Model identifies six key elements for effective chronic disease management: self-management support, delivery system design, decision support, clinical information systems, health system leadership, and community linkages. It shifts care from reactive acute treatment to proactive population management.

#85 · Care Management

What is care management for COPD patients?

Answer

COPD case management focuses on smoking cessation, inhaler technique education, pulmonary rehabilitation referral, action plan development for exacerbations, and vaccination compliance. Recognizing early exacerbation warning signs and having a clear action plan reduces hospitalizations and improves quality of life.

#86 · Care Management

What is a disease-specific action plan?

Answer

A disease-specific action plan gives the patient clear instructions for what to do when symptoms worsen. For heart failure, this means who to call when weight increases by a defined amount. For asthma, it means the stepwise response to worsening peak flow. Action plans reduce emergency department visits.

#87 · Care Management

What is care management in workers' compensation?

Answer

Workers' compensation case management coordinates medical treatment for work-related injuries, facilitates communication between the injured worker, employer, and insurer, and focuses on return to work. The case manager advocates for appropriate medical care while also supporting the employer's goal of minimizing lost work time.

#88 · Care Management

What is care management in occupational health?

Answer

Occupational health case management addresses work-related illness and injury, disability management, FMLA coordination, fitness-for-duty evaluation, and return-to-work planning. It requires knowledge of occupational health regulations, the ADA, workers' compensation systems, and functional capacity assessment.

#89 · Care Management

What is care management for pediatric populations?

Answer

Pediatric case management involves the family as central partners and addresses developmental needs, school and community supports, transition to adult care, and family caregiver education. Special attention is given to children with complex medical needs, developmental delays, and chronic conditions.

#90 · Care Management

What is care management for patients with mental illness?

Answer

Mental health case management coordinates psychiatric care, medication management, community support, housing, employment, and social connection for patients with serious mental illness. It applies a recovery-oriented approach that emphasizes the patient's strengths and life goals rather than symptom management alone.

#91 · Care Management

What is a high-utilizer?

Answer

A high-utilizer is a patient with frequent emergency department visits and hospitalizations, often driven by unmet social needs, behavioral health conditions, inadequate primary care access, or poor chronic disease management. Intensive case management for high-utilizers can dramatically reduce costs and improve outcomes.

#92 · Care Management

What is the return on investment (ROI) of case management?

Answer

ROI is demonstrated by comparing the cost of the case management program to the savings from reduced hospitalizations, ED visits, and avoidable procedures. A single prevented hospitalization often covers the annual cost of the program, making case management a financially sound investment in managed care.

#93 · Care Management

What clinical outcomes does case management track?

Answer

Case management tracks hospitalization rates, ED visit rates, 30-day readmissions, disease control metrics (A1c, blood pressure), medication adherence, and patient-reported outcomes. These measures demonstrate the clinical impact of the program on the enrolled population.

#94 · Care Management

What is telehealth case management?

Answer

Telehealth case management uses video visits, remote monitoring devices, and telephonic contact to provide case management services to patients regardless of geographic location. It improves access for rural and underserved patients but requires addressing technology access barriers and payer coverage rules.

#95 · Care Management

What is remote patient monitoring?

Answer

Remote patient monitoring uses digital devices to collect health data (weight, blood pressure, blood glucose) from patients at home and transmit it to the care team for review. It allows early identification of deterioration and can reduce hospitalizations for heart failure, COPD, and diabetes.

#96 · Care Management

What is the role of the case manager in end-of-life care?

Answer

The case manager in end-of-life care facilitates goals-of-care conversations, ensures advance directives are documented and accessible, coordinates hospice or palliative care referrals, supports the patient and family through the dying process, and addresses practical needs such as durable medical equipment and caregiver support.

#97 · Care Management

What is the role of the case manager in supporting family caregivers?

Answer

The case manager assesses caregiver burden, provides education and training, connects caregivers with respite care and support groups, and advocates for services that support the caregiver's ability to continue in the caregiving role. Caregiver health directly affects the patient's ability to remain safely at home.

#98 · Care Management

What is the role of care management in reducing health disparities?

Answer

The case manager identifies disparities in care access, quality, and outcomes for patients from marginalized groups and takes proactive steps to address them through advocacy, culturally responsive communication, connection to community resources, and challenging discriminatory practices in the healthcare system.

#99 · Care Management

What is health equity?

Answer

Health equity means every person has a fair and just opportunity to be as healthy as possible. Achieving it requires removing avoidable disparities rooted in social injustice, including unequal access to care, differential quality of care, and social conditions that make health harder to achieve for some groups.

#100 · Care Management

What is the relationship between housing instability and health?

Answer

Housing instability is associated with poor chronic disease management, medication non-adherence, inability to follow dietary restrictions, increased stress, and frequent emergency department use. Stable housing is the foundational social determinant that must be in place before other health interventions can be sustained.

#101 · Care Management

What is care management for patients with HIV/AIDS?

Answer

HIV case management coordinates antiretroviral therapy, lab monitoring, adherence support, mental health care, substance use treatment, and social services. Adherence to ART is essential for viral suppression and prevention of transmission, making medication adherence support a central case management function.

#102 · Care Management

What is care management for patients with chronic kidney disease?

Answer

CKD case management monitors kidney function, manages cardiovascular risk, adjusts medications for renal dosing, provides dietary education on protein and potassium intake, and prepares patients for renal replacement therapy options including dialysis and transplant well before end-stage disease.

#103 · Care Management

What is care management in the emergency department?

Answer

ED case management identifies patients with high-risk social situations or frequent ED use, connects them to community resources and primary care, facilitates disposition decisions, and coordinates follow-up care to address the root causes of ED utilization and reduce unnecessary returns.

#104 · Care Management

What is the role of the case manager in identifying abuse and neglect?

Answer

The case manager screens for signs of physical, emotional, sexual, and financial abuse, and neglect across all patient populations. Mandatory reporting laws require reporting of suspected child abuse and elder abuse to designated authorities, regardless of the patient's wishes or the family's explanation.

#105 · Care Management

What is the role of case management in domestic violence situations?

Answer

When a patient discloses domestic violence, the case manager assesses current safety, connects the patient with domestic violence resources, and in most states allows the patient to make their own decision about reporting. Unlike child or elder abuse, adult domestic violence reporting is generally not mandated.

#106 · Care Management

What is the role of case management in suicide risk?

Answer

When a patient expresses suicidal ideation, the case manager conducts an immediate safety assessment including asking directly about plan and means, follows the organization's suicide risk protocol, and connects the patient to appropriate mental health services. Warning signs such as giving away belongings require immediate attention.

#107 · Care Management

What is care management for patients with Alzheimer's disease?

Answer

Dementia case management addresses cognitive and functional decline, safety in the home environment, caregiver education and support, driving safety, advance care planning, and coordination with memory care specialists. The family and caregiver are central partners in the care plan.

#108 · Care Management

What is care management in a managed care organization?

Answer

In a managed care organization, the case manager reviews claims and clinical data to identify high-risk members, manages complex cases, coordinates care across the network, supports prior authorization processes, and tracks quality metrics. The case manager must manage the inherent tension between cost containment and patient advocacy.

#109 · Care Management

What is the role of the case manager in reducing polypharmacy?

Answer

The case manager reviews the patient's complete medication list for duplications, interactions, and medications no longer needed, and advocates for medication simplification through the prescribing team. Polypharmacy increases fall risk, cognitive impairment, and adverse drug events, particularly in older adults.

#110 · Care Management

What is the role of case management in post-acute care?

Answer

Post-acute case management ensures that patients discharged to SNF, IRF, home health, or LTACH receive the level of care that matches their clinical needs, progress is monitored, and transition back to the community is planned early in the post-acute stay.

#111 · Care Management

What is care management in the neonatal intensive care unit (NICU)?

Answer

NICU case management supports families through the high-stress NICU experience, coordinates multidisciplinary care for medically complex newborns, plans discharge to home or specialized care, and connects families with early intervention, developmental follow-up, and community support resources.

#112 · Care Management

What is care management for patients with spinal cord injury?

Answer

SCI case management is complex and lifelong, addressing medical complications (pressure injuries, autonomic dysreflexia, neurogenic bladder and bowel), equipment needs, home modification, attendant care, vocational rehabilitation, and community reintegration across a lifetime of disability.

#113 · Care Management

What is care management in hospice?

Answer

Hospice case management coordinates comfort-focused medical care, pain and symptom management, spiritual care, social work, and bereavement support for dying patients and their families. Medicare Hospice Benefit covers these services for patients with a prognosis of six months or less who choose comfort-focused care.

#114 · Care Management

What is the role of case management in reducing avoidable ED visits?

Answer

Case managers identify the root causes of ED utilization, which often include unmet primary care needs, medication non-adherence, behavioral health crises, and social instability. Targeted interventions addressing these root causes, rather than the ED visit itself, reduce unnecessary utilization.

#115 · Care Management

What is care management documentation?

Answer

Case management documentation records assessments, care plans, interventions, patient contacts, monitoring findings, and case closure rationale. It must be timely, accurate, and complete because it serves as the legal record of services provided and the basis for quality and outcome reporting.

#116 · Reimbursement Methods

What is Medicare Part A?

Answer

Medicare Part A covers inpatient hospital care, skilled nursing facility care after a qualifying three-day hospital stay, home health, and hospice. Most beneficiaries receive Part A at no premium because they or their spouse paid Medicare taxes during working years.

#117 · Reimbursement Methods

What is Medicare Part B?

Answer

Medicare Part B covers outpatient medical services including physician visits, durable medical equipment, preventive services, and outpatient therapy. Beneficiaries pay a monthly premium and a 20% coinsurance after meeting the annual deductible.

#118 · Reimbursement Methods

What is Medicare Part C (Medicare Advantage)?

Answer

Medicare Part C, also called Medicare Advantage, is an alternative to Original Medicare where beneficiaries enroll in a private health plan that must cover at least the same benefits as Parts A and B. Plans may offer additional benefits and often include Part D drug coverage.

#119 · Reimbursement Methods

What is Medicare Part D?

Answer

Medicare Part D covers outpatient prescription drugs through private insurance plans approved by CMS. Beneficiaries enroll in a standalone Part D plan or receive drug coverage through a Medicare Advantage plan. Coverage and costs vary by plan, formulary, and drug tier.

#120 · Reimbursement Methods

What is the Medicare three-day rule for SNF coverage?

Answer

Medicare covers skilled nursing facility care only after the patient has had a qualifying three-day inpatient hospital stay (not observation status). Observation days do not count toward this requirement, a distinction that can surprise patients who were not admitted as inpatients.

#121 · Reimbursement Methods

What is observation status?

Answer

Observation status is a hospital billing classification where patients receive outpatient services while being monitored in the hospital. Observation days do not count toward the Medicare three-day inpatient requirement for SNF coverage, and patients may owe higher cost-sharing for medications received during observation stays.

#122 · Reimbursement Methods

What is the Medicare home health benefit?

Answer

Medicare Part A or B covers home health when the patient is homebound (leaving home requires considerable and taxing effort) and has an active skilled need such as skilled nursing, physical therapy, or speech therapy. Aide services alone do not qualify without a skilled need.

#123 · Reimbursement Methods

What is the Medicare hospice benefit?

Answer

The Medicare Hospice Benefit covers comfort-focused medical care, nursing, social work, spiritual care, and bereavement support for beneficiaries with a prognosis of six months or less who choose comfort-focused care. Patients waive coverage for curative treatment of the terminal diagnosis.

#124 · Reimbursement Methods

What is the DRG payment system?

Answer

Diagnosis-Related Groups (DRGs) are the Medicare inpatient hospital payment system where hospitals receive a fixed payment based on the patient's principal diagnosis, regardless of actual length of stay or resource use. Complications and comorbidities may increase the DRG weight and payment.

#125 · Reimbursement Methods

What is a Medicare outlier payment?

Answer

When the total cost of an inpatient case significantly exceeds the DRG payment threshold, Medicare provides an outlier payment in addition to the DRG to partially offset the unusually high cost. It is designed to share financial risk for extraordinarily expensive cases.

#126 · Reimbursement Methods

What is the Medicare Inpatient Rehabilitation Facility (IRF) payment system?

Answer

IRF stays are paid under a prospective payment system based on the patient's functional status on admission (measured by the IRF-PAI instrument) and diagnosis. To qualify, patients must need and be able to participate in at least three hours of intensive therapy per day.

#127 · Reimbursement Methods

What is the Medicare LTACH payment system?

Answer

Long-Term Acute Care Hospitals are paid under a prospective payment system for patients requiring prolonged acute-level care, most commonly for ventilator weaning. The average LTACH length of stay must exceed 25 days. CMS has specific clinical criteria for appropriate LTACH use.

#128 · Reimbursement Methods

What is a fee-for-service payment model?

Answer

In fee-for-service, providers are paid separately for each service they provide, creating an incentive to deliver more services. This model does not reward quality or efficiency, which is why value-based payment models were developed to better align financial incentives with patient outcomes.

#129 · Reimbursement Methods

What is capitation?

Answer

Capitation pays a health plan or provider group a fixed amount per member per month regardless of the services used. It transfers financial risk to the provider and creates an incentive to invest in prevention and care management that reduce expensive acute care utilization.

#130 · Reimbursement Methods

What is a Medicare Shared Savings Program?

Answer

The Medicare Shared Savings Program allows ACOs to keep a portion of savings they generate for Medicare by delivering more efficient, coordinated care. ACOs that reduce spending while meeting quality benchmarks receive a share of the savings, creating a financial incentive for care management investment.

#131 · Reimbursement Methods

What is value-based purchasing?

Answer

Value-based purchasing is a CMS payment approach that adjusts hospital and provider payments based on quality metrics and patient outcomes. Hospitals receive bonuses for high performance and penalties for poor performance on measures including patient experience, clinical process, and outcomes.

#132 · Reimbursement Methods

What is Medicaid?

Answer

Medicaid is a joint federal-state health insurance program for people with low income, including children, pregnant women, adults, elderly, and individuals with disabilities. States administer Medicaid within federal guidelines and may expand or restrict optional benefits. Eligibility and benefits vary by state.

#133 · Reimbursement Methods

What is the difference between Medicare and Medicaid?

Answer

Medicare is a federal insurance program primarily for people 65 and older and those with qualifying disabilities, based on work history. Medicaid is a joint federal-state program based on income and other eligibility criteria, primarily serving low-income individuals. A person can qualify for both, making them dual-eligible.

#134 · Reimbursement Methods

What is a dual-eligible beneficiary?

Answer

A dual-eligible beneficiary qualifies for both Medicare and Medicaid. Medicare is the primary payer for services within its scope; Medicaid covers cost-sharing and services Medicare does not cover. Dual-eligible patients often have complex needs and are a high priority for case management programs.

#135 · Reimbursement Methods

What is Medicaid managed care?

Answer

In Medicaid managed care, states contract with private health plans to deliver Medicaid benefits in exchange for a capitated payment. Plans assume financial risk and are expected to manage costs through care coordination, utilization management, and quality improvement programs.

#136 · Reimbursement Methods

What is CHIP (Children's Health Insurance Program)?

Answer

CHIP provides low-cost health coverage to children in families with incomes too high to qualify for Medicaid but who cannot afford private insurance. States administer CHIP and may integrate it with Medicaid or operate it as a separate program with different benefit packages.

#137 · Reimbursement Methods

What is a Health Maintenance Organization (HMO)?

Answer

An HMO is a type of managed care plan that requires members to select a primary care physician who coordinates all care and provides referrals to specialists within the network. Out-of-network services are generally not covered except in emergencies.

#138 · Reimbursement Methods

What is a Preferred Provider Organization (PPO)?

Answer

A PPO is a managed care plan that provides coverage for both in-network and out-of-network services, with lower cost-sharing for in-network use. Members do not need a primary care physician or referrals to see specialists.

#139 · Reimbursement Methods

What is a Point-of-Service (POS) plan?

Answer

A POS plan is a hybrid managed care plan that combines elements of HMO and PPO. Members choose a primary care physician within the network and need referrals for specialist visits, but can seek out-of-network care at higher cost-sharing.

#140 · Reimbursement Methods

What is an Exclusive Provider Organization (EPO)?

Answer

An EPO is a managed care plan that requires members to use providers within the network for all non-emergency care. Like an HMO, out-of-network services are not covered, but like a PPO, no primary care physician or referral is required.

#141 · Reimbursement Methods

What is a network adequacy requirement?

Answer

Network adequacy requirements mandate that health plans maintain sufficient numbers of in-network providers to ensure members can access covered services within a reasonable time and distance. When in-network providers are unavailable, plans may be required to authorize out-of-network care at in-network cost-sharing.

#142 · Reimbursement Methods

What is balance billing?

Answer

Balance billing occurs when a provider bills a patient for the difference between the provider's charge and what the insurance paid. The No Surprises Act protects patients from unexpected balance bills from out-of-network providers in situations where they could not reasonably have chosen an in-network provider.

#143 · Reimbursement Methods

What is the No Surprises Act?

Answer

The No Surprises Act protects patients from unexpected out-of-network bills, particularly from emergency services and out-of-network providers at in-network facilities. It limits patient cost-sharing to the in-network amount in qualifying situations regardless of whether the provider was in-network.

#144 · Reimbursement Methods

What is the Mental Health Parity and Addiction Equity Act?

Answer

The Mental Health Parity and Addiction Equity Act prohibits health plans from applying more restrictive coverage criteria to behavioral health conditions than to comparable medical or surgical conditions. Case managers should flag denials that apply stricter criteria to behavioral health than to equivalent medical benefits.

#145 · Reimbursement Methods

What is a medical loss ratio (MLR)?

Answer

The medical loss ratio is the percentage of premium revenue a health plan spends on medical care and quality improvement activities versus administrative costs and profits. The ACA requires most plans to spend at least 80-85% of premiums on medical care; plans that fall short must rebate the difference to members.

#146 · Reimbursement Methods

What is a prior authorization denial?

Answer

A prior authorization denial occurs when the plan determines the requested service does not meet coverage criteria. The case manager should review the denial rationale, gather additional supporting clinical documentation, and pursue an appeal or peer-to-peer review when the denial appears clinically inappropriate.

#147 · Reimbursement Methods

What is the appeals process for insurance denials?

Answer

The appeals process allows patients and providers to challenge denied claims or prior authorization decisions. An internal appeal is reviewed by the plan; if denied, an external appeal may be requested from an independent organization. Exhausting all appeal levels is required before most regulatory escalations.

#148 · Reimbursement Methods

What is an Independent Medical Review (IMR)?

Answer

An Independent Medical Review is an external appeal reviewed by an independent clinical reviewer not affiliated with the health plan. It is typically available after an internal appeal is exhausted. IMR decisions are binding on the plan in most states, making it an important patient advocacy tool.

#149 · Reimbursement Methods

What is COBRA?

Answer

COBRA (Consolidated Omnibus Budget Reconciliation Act) allows employees and their dependents to continue employer-sponsored health coverage for up to 18-36 months after a qualifying event such as job loss, reduction in hours, or divorce. The individual pays the full premium, which can be expensive.

#150 · Reimbursement Methods

What is a Special Enrollment Period (SEP)?

Answer

A Special Enrollment Period allows individuals to enroll in or change health insurance outside of the annual open enrollment period following a qualifying life event such as losing other coverage, getting married, having a baby, or aging off a parent's insurance at age 26.

#151 · Reimbursement Methods

What is the Affordable Care Act (ACA)?

Answer

The ACA expanded health insurance coverage through Medicaid expansion, insurance marketplace subsidies, and insurance market reforms including prohibition of lifetime limits, coverage of pre-existing conditions, and required coverage of essential health benefits including mental health and preventive services.

#152 · Reimbursement Methods

What are essential health benefits under the ACA?

Answer

Essential health benefits are ten categories that all ACA marketplace plans must cover: ambulatory services, emergency services, hospitalization, maternity care, mental health and substance use, prescription drugs, rehabilitation, laboratory services, preventive services, and pediatric services.

#153 · Reimbursement Methods

What is a formulary?

Answer

A formulary is the health plan's list of covered prescription drugs, organized into tiers with different cost-sharing levels. Lower-tier drugs (generic, preferred brand) have lower cost-sharing; higher-tier drugs (specialty, non-preferred brand) have higher cost-sharing. Non-formulary drugs may require prior authorization.

#154 · Reimbursement Methods

What is a specialty drug?

Answer

Specialty drugs are high-cost medications typically requiring special handling, administration, or monitoring, such as biologics, chemotherapy agents, and immunomodulators. They typically require prior authorization and may have step therapy requirements before the plan will approve them.

#155 · Reimbursement Methods

What is a pharmaceutical manufacturer patient assistance program?

Answer

Pharmaceutical manufacturer patient assistance programs provide free or discounted medications to eligible patients who cannot afford them. Case managers help patients apply for these programs when cost is a barrier to medication adherence. Copay assistance cards are available for many brand medications.

#156 · Reimbursement Methods

What is anti-kickback statute relevance to copay cards?

Answer

Federal anti-kickback provisions prohibit the use of manufacturer copay assistance cards when the patient has Medicaid coverage, because the arrangement effectively eliminates patient cost-sharing while the government payer continues to pay full drug cost. This constitutes prohibited remuneration when Medicaid is involved.

#157 · Reimbursement Methods

What is the Medicare Annual Wellness Visit?

Answer

The Medicare Annual Wellness Visit is a preventive service covered under Part B at no cost to the beneficiary. It includes health risk assessment, review of medications and preventive services, and development of a personalized prevention plan. It cannot include a physical examination but may be billed alongside a separately documented E&M; service.

#158 · Reimbursement Methods

What are Hospital-Acquired Conditions (HACs)?

Answer

HACs are conditions that develop during a hospital stay that are reasonably preventable with evidence-based care. CMS does not provide additional payment for treating HACs, creating a financial incentive for prevention. Examples include stage 3 and 4 pressure injuries, catheter-associated UTIs, and surgical site infections.

#159 · Reimbursement Methods

What is the CMS Inpatient Prospective Payment System (IPPS)?

Answer

The IPPS pays hospitals a fixed amount for Medicare inpatient stays based on the DRG, adjusted for hospital characteristics. Hospitals profit when costs are below the DRG payment and lose money when costs exceed it, creating an incentive for efficient, evidence-based inpatient care management.

#160 · Reimbursement Methods

What is a Medicare Administrative Contractor (MAC)?

Answer

MACs are private companies that process Medicare fee-for-service claims on behalf of CMS. They also conduct audits, issue coverage policies, and provide education to providers. The case manager may interact with MACs when dealing with claims disputes or coverage determinations.

#161 · Reimbursement Methods

What is Recovery Audit Contractor (RAC) auditing?

Answer

RACs are CMS contractors that identify and recover improper Medicare and Medicaid payments by reviewing claims for billing errors, coding issues, and medical necessity. Hospitals and providers that fail documentation standards may be required to return overpayments, creating a risk management incentive for thorough case management documentation.

#162 · Reimbursement Methods

What is the Low Income Subsidy (LIS) / Extra Help?

Answer

The Low Income Subsidy, also called Extra Help, is a Medicare program that significantly reduces Part D premiums, deductibles, and copayments for beneficiaries who qualify based on income and resources. Case managers should evaluate all Medicare beneficiaries with high drug costs for LIS eligibility.

#163 · Reimbursement Methods

What is the Medicare Part D coverage gap (donut hole)?

Answer

The Medicare Part D coverage gap was a temporary limit on drug coverage where beneficiaries paid a higher share of drug costs after total spending exceeded an initial coverage limit. The ACA gradually closed the donut hole, and beneficiaries now pay no more than 25% of drug costs in the coverage gap.

#164 · Reimbursement Methods

What is ERISA and its impact on health benefits?

Answer

ERISA (Employee Retirement Income Security Act) governs most self-funded employer health plans and preempts many state insurance laws. Self-funded ERISA plans are not subject to state insurance mandates, which can affect benefits available to employees in employer-sponsored plans.

#165 · Reimbursement Methods

What is a self-funded health plan?

Answer

A self-funded (self-insured) employer health plan pays claims directly from employer funds rather than purchasing traditional insurance. The employer assumes the financial risk and contracts with a third-party administrator to process claims. Most large employers self-fund their health benefits.

#166 · Reimbursement Methods

What is stop-loss insurance?

Answer

Stop-loss insurance protects self-funded employers against catastrophically large claims by providing reimbursement when claims exceed a specified threshold at the individual or aggregate level. It allows smaller employers to self-fund without unlimited financial exposure.

#167 · Reimbursement Methods

What is FMLA and how does it relate to case management?

Answer

The Family and Medical Leave Act allows eligible employees to take up to 12 weeks of unpaid leave per year for serious health conditions, childbirth, or military situations. Case managers in occupational health and workers' compensation may coordinate FMLA leave with medical treatment and return-to-work planning.

#168 · Reimbursement Methods

What is Medicare Secondary Payer (MSP)?

Answer

Medicare Secondary Payer rules determine when Medicare pays first and when it is secondary to another insurer. For working-aged beneficiaries with employer coverage, the employer plan is typically primary. For ESRD beneficiaries, Medicare becomes primary after a coordination period. MSP violations carry significant penalties.

#169 · Reimbursement Methods

What is coordination of benefits (COB)?

Answer

Coordination of benefits determines which insurance plan pays first when a patient has more than one coverage source. The primary plan pays its portion first; the secondary plan pays some or all of the remaining amount. COB prevents double payment and reduces unnecessary duplication of benefits.

#170 · Reimbursement Methods

What is the birthday rule in COB for dependents?

Answer

The birthday rule is a common COB method for determining which parent's plan is primary for a dependent child. The plan of the parent whose birthday falls earlier in the calendar year is primary; if both parents have the same birthday, the plan in force longer is primary.

#171 · Reimbursement Methods

What is Workers' Compensation insurance?

Answer

Workers' Compensation provides wage replacement and medical benefits to employees injured on the job, regardless of fault. It is state-regulated and mandatory for most employers. When workers' compensation applies to an injury, it is primary over other health insurance for care related to the work injury.

#172 · Reimbursement Methods

What is a preferred drug list?

Answer

A preferred drug list (PDL) is a subset of the formulary listing medications the plan prefers because of clinical effectiveness, safety, and cost. Drugs on the preferred list have lower cost-sharing; non-preferred drugs have higher cost-sharing. PDLs are often managed through state Medicaid programs.

#173 · Reimbursement Methods

What is a coverage determination?

Answer

A coverage determination is the health plan's decision about whether a service is covered for a specific patient. For Medicare, the initial coverage determination is made by the plan or MAC; patients and providers have the right to appeal adverse determinations through a structured appeals process.

#174 · Reimbursement Methods

What is utilization review?

Answer

Utilization review is the systematic evaluation of healthcare services to ensure they are medically necessary, appropriate to the clinical situation, and provided at the right level of care. It includes prospective review (prior authorization), concurrent review (inpatient monitoring), and retrospective review (claims audit).

#175 · Reimbursement Methods

What is InterQual?

Answer

InterQual is a widely used clinical decision support tool that provides evidence-based criteria for medical necessity determinations across care settings. Payers and hospitals use InterQual to guide utilization review decisions for inpatient admissions, level of care transitions, and post-acute placements.

#176 · Reimbursement Methods

What is Milliman Care Guidelines?

Answer

Milliman Care Guidelines (MCG) is another widely used clinical criteria tool used by health plans and hospitals to evaluate medical necessity and appropriate level of care. Like InterQual, MCG provides evidence-based criteria but case managers must still apply clinical judgment alongside the criteria.

#177 · Reimbursement Methods

What is a clinical practice guideline and its role in coverage decisions?

Answer

Clinical practice guidelines are systematically developed recommendations based on evidence review. Payers use guidelines to develop coverage policies; treating physicians use them to guide care. When a denial conflicts with published guidelines, the treating physician can cite the guidelines in a peer-to-peer review.

#178 · Reimbursement Methods

What is the role of the physician advisor in utilization management?

Answer

The physician advisor is a licensed physician who reviews complex utilization management decisions, including clinical disputes about level of care and medical necessity. When a case manager believes a denial is clinically inappropriate, the physician advisor reviews the clinical information and can engage in peer-to-peer review.

#179 · Reimbursement Methods

What is the difference between inpatient and observation status for billing?

Answer

Inpatient status is a formal admission billed under Medicare Part A. Observation status is an outpatient service billed under Part B with different cost-sharing. Observation days do not count toward the three-day SNF qualifying stay, making the distinction clinically and financially significant for Medicare patients.

#180 · Reimbursement Methods

What is a Medicare Advantage Special Needs Plan (SNP)?

Answer

Medicare Advantage SNPs are specialized plans designed for specific populations including dual-eligible beneficiaries (D-SNPs), institutionalized individuals (I-SNPs), and people with specific chronic conditions (C-SNPs). They offer targeted benefits and coordinated care for their specific population.

#181 · Reimbursement Methods

What is managed long-term services and supports (MLTSS)?

Answer

MLTSS integrates long-term care services including home and community-based services and nursing facility care into a managed care framework, with a single capitated payment covering all services. It promotes community living, integrates care coordination, and creates accountability for outcomes.

#182 · Reimbursement Methods

What is a Medicaid waiver?

Answer

Medicaid waivers allow states to test innovative approaches to coverage and service delivery that differ from standard Medicaid rules. Common waivers include HCBS waivers (1915c) providing community-based services and comprehensive demonstration waivers (1115) that expand eligibility or restructure benefits.

#183 · Reimbursement Methods

What is the Area Agency on Aging (AAA)?

Answer

Area Agencies on Aging are local organizations funded under the Older Americans Act that coordinate community services for older adults including meal programs, transportation, caregiver support, and legal assistance. They are an important community resource for case managers working with older adult populations.

#184 · Reimbursement Methods

What is PACE (Program of All-inclusive Care for the Elderly)?

Answer

PACE is a Medicare and Medicaid program serving people age 55 and older who need nursing facility level of care but can safely live in the community with comprehensive support. PACE provides all medical and social services through an interdisciplinary team and adult day center model.

#185 · Reimbursement Methods

What is community benefit in nonprofit hospitals?

Answer

Nonprofit hospitals must provide community benefit in exchange for their tax-exempt status. This includes charity care, free clinics, health education, research, and the required Community Health Needs Assessment every three years. Case managers working in nonprofit hospitals may be involved in community benefit programs.

#186 · Reimbursement Methods

What is financial assistance and charity care?

Answer

Financial assistance programs, also called charity care, provide free or discounted care to uninsured or underinsured patients who cannot afford to pay. Nonprofit hospitals are required to have a financial assistance policy. Case managers help eligible patients apply for these programs during care planning.

#187 · Reimbursement Methods

What is a Health Savings Account (HSA)?

Answer

An HSA is a tax-advantaged account that allows individuals enrolled in a high-deductible health plan to save pre-tax dollars for qualified medical expenses. Funds roll over year to year and can be invested. HSAs reduce the tax burden of healthcare costs and can be used in retirement for medical expenses.

#188 · Reimbursement Methods

What is Medicare's Annual Wellness Visit versus a physical exam?

Answer

The Annual Wellness Visit is a preventive service that creates a health risk profile and personalized prevention plan; it is not a physical exam. If the physician conducts a separate evaluation and management service for a chronic condition during the same visit, both services can be billed separately with appropriate documentation.

#189 · Reimbursement Methods

What is bundled payment?

Answer

Bundled payment provides a single payment for all services associated with a defined episode of care, such as a hip replacement including the surgery, hospitalization, rehabilitation, and follow-up. All providers share a fixed payment, creating an incentive for coordination and efficiency.

#190 · Reimbursement Methods

What is pay-for-performance?

Answer

Pay-for-performance (P4P) programs reward providers and health plans that achieve specific quality or efficiency benchmarks with bonus payments or avoid penalties for poor performance. It is one approach to value-based payment that links financial incentives to measurable outcomes.

#191 · Reimbursement Methods

What is the Medicare Quality Payment Program (QPP)?

Answer

The QPP is CMS's framework for rewarding Medicare clinicians for quality and value rather than volume. Clinicians participate either through MIPS (Merit-based Incentive Payment System) or by joining an Advanced Alternative Payment Model, with financial adjustments based on performance.

#192 · Reimbursement Methods

What are CMS star ratings?

Answer

CMS publishes star ratings for Medicare Advantage and Part D plans based on quality and performance measures. Higher-rated plans receive higher reimbursement and are more attractive to beneficiaries. Case managers working in managed care are aware of star ratings as a driver of quality program investment.

#193 · Reimbursement Methods

What is the Non-Emergency Medical Transportation (NEMT) benefit?

Answer

NEMT is a Medicaid benefit that provides transportation to and from medical appointments for beneficiaries who have no other means of transportation. Case managers use NEMT to remove transportation as a barrier to care for Medicaid patients, particularly in rural areas.

#194 · Reimbursement Methods

What is an LTACH and who qualifies?

Answer

Long-Term Acute Care Hospitals serve medically complex patients who require prolonged acute-level care, most commonly those being weaned from mechanical ventilation or managing complex wounds and organ dysfunction. The average length of stay must exceed 25 days, and specific CMS criteria determine appropriate admission.

#195 · Reimbursement Methods

What is the three-day payment rule for Medicare home health?

Answer

Medicare home health does not require a prior hospital stay; it is available to homebound patients who have an active skilled need and are under a physician's plan of care. The common misconception that a hospital stay is required for home health confuses the home health requirement with the SNF three-day rule.

#196 · Reimbursement Methods

What is DME (Durable Medical Equipment) coverage under Medicare?

Answer

Medicare Part B covers DME with a physician's order and certificate of medical necessity. DME must be medically necessary, durable, used for a medical purpose, not generally useful to healthy persons, and appropriate for home use. The case manager arranges DME through an in-network supplier before discharge.

#197 · Reimbursement Methods

What is the impact of Medicaid expansion under the ACA?

Answer

The ACA allowed states to expand Medicaid to adults with incomes up to 138% of the federal poverty level. States that expanded Medicaid saw significant reductions in uninsured rates, hospital uncompensated care costs, and financial toxicity. As of 2025, most but not all states have adopted the expansion.

#198 · Reimbursement Methods

What is a benchmark in reimbursement terms?

Answer

In managed care, a benchmark is a reference level of spending or quality performance used to calculate bonuses or penalties. CMS sets benchmarks for ACO and Medicare Advantage programs; plans that spend below the benchmark while meeting quality thresholds share in the savings.

#199 · Reimbursement Methods

What is the role of the case manager in identifying insurance coverage gaps?

Answer

The case manager proactively identifies coverage gaps that could affect the patient's ability to access needed services. This includes checking formulary coverage, network adequacy, prior authorization requirements, and cost-sharing levels. Addressing these gaps before discharge or treatment initiation prevents care delays and readmissions.

#200 · Reimbursement Methods

What is the Veterans Administration (VA) health system?

Answer

The VA provides healthcare to eligible veterans through its own network of hospitals and clinics. Veterans may also be eligible for VA-covered community care when VA services are not available within access standards. Case managers working with veterans assess VA eligibility as part of the resource identification process.

#201 · Psychosocial Concepts and Support Systems

What is the biopsychosocial model in case management assessment?

Answer

The biopsychosocial model recognizes that health is determined by the interaction of biological (disease, genetics), psychological (emotions, cognition, behavior), and social (relationships, environment, resources) factors. Case management assessment must address all three dimensions to develop an effective, individualized care plan.

#202 · Psychosocial Concepts and Support Systems

What is a psychosocial assessment?

Answer

A psychosocial assessment evaluates the patient's mental health, emotional state, cognitive function, coping style, social support, family dynamics, cultural background, spiritual needs, financial situation, housing stability, and barriers to care. It provides the social and psychological context for the clinical care plan.

#203 · Psychosocial Concepts and Support Systems

What is adjustment disorder?

Answer

Adjustment disorder is an emotional or behavioral response to an identifiable stressor, such as a new diagnosis, that is out of proportion to or significantly impacts functioning. It is common after major health events and responds well to supportive counseling, psychoeducation, and practical support.

#204 · Psychosocial Concepts and Support Systems

What is the PHQ-9 and when is it used?

Answer

The PHQ-9 is a validated nine-item depression screening tool. Scores range from 0 to 27; scores of 5-9 indicate mild depression, 10-14 moderate, 15-19 moderately severe, and 20+ severe. A score of 10 or more typically warrants further evaluation and intervention. Case managers use it to identify depression affecting self-care.

#205 · Psychosocial Concepts and Support Systems

What is the GAD-7 and when is it used?

Answer

The GAD-7 is a validated seven-item anxiety screening tool. Scores of 5, 10, and 15 represent mild, moderate, and severe anxiety. It is used in primary care and case management settings to identify anxiety disorders that may be contributing to somatic symptoms or interfering with chronic disease management.

#206 · Psychosocial Concepts and Support Systems

What is the relationship between depression and chronic disease?

Answer

Depression significantly worsens outcomes in most chronic conditions including diabetes, heart disease, COPD, and cancer. It reduces medication adherence, self-care behaviors, and engagement with the healthcare system. Treating depression concurrently with the chronic disease improves both mental and physical health outcomes.

#207 · Psychosocial Concepts and Support Systems

What is caregiver burden?

Answer

Caregiver burden is the physical, emotional, social, and financial strain experienced by family members who provide care to a loved one with a health condition or disability. Caregiver burnout is common and directly threatens the sustainability of home-based care arrangements, making caregiver assessment a clinical priority.

#208 · Psychosocial Concepts and Support Systems

What is social support and how does it affect health?

Answer

Social support includes emotional support, practical assistance, information, and feedback from relationships. Strong social support is associated with better chronic disease management, medication adherence, and overall health outcomes. Social isolation is an independent risk factor for poor health and premature death.

#209 · Psychosocial Concepts and Support Systems

What is the grief process and how does it relate to illness?

Answer

Grief is not limited to death; patients and families grieve losses of function, identity, independence, and future plans associated with serious illness. Case managers recognize grief responses, provide space for emotional expression, and ensure patients have access to appropriate psychological support without pathologizing normal grief.

#210 · Psychosocial Concepts and Support Systems

What is complicated grief?

Answer

Complicated grief (also called prolonged grief disorder) occurs when the grief process is prolonged and functionally impairing beyond typical grief parameters. It may follow bereavement or significant loss from illness and requires specialized mental health intervention beyond standard supportive counseling.

#211 · Psychosocial Concepts and Support Systems

What is fear of recurrence in cancer survivors?

Answer

Fear of cancer recurrence is a named clinical concern in cancer survivorship, affecting a large proportion of survivors and warranting targeted psychological support. It is distinct from survivor's guilt and represents a persistent worry about cancer returning that can significantly impair quality of life and healthcare engagement.

#212 · Psychosocial Concepts and Support Systems

What is the role of case management in post-traumatic stress disorder (PTSD)?

Answer

PTSD in medical patients may result from trauma associated with illness, medical procedures, or prior life events. Case managers screen for PTSD using tools like the PC-PTSD-5, recognize trauma responses that affect healthcare engagement, and coordinate care with mental health providers using trauma-informed approaches.

#213 · Psychosocial Concepts and Support Systems

What is post-intensive care syndrome (PICS)?

Answer

PICS describes the cluster of new or worsening cognitive, psychological, and physical impairments that persist after critical illness and ICU hospitalization. Patients experience cognitive impairment, depression, PTSD, and physical weakness. Case managers address PICS through coordinated post-discharge follow-up across rehabilitation and mental health services.

#214 · Psychosocial Concepts and Support Systems

What is Erikson's theory of psychosocial development and its relevance to case management?

Answer

Erikson's theory describes developmental tasks across the lifespan, each representing a psychosocial crisis to be resolved. Understanding a patient's developmental stage helps the case manager contextualize their responses to illness, loss, and care decisions, particularly in older adults navigating ego integrity versus despair.

#215 · Psychosocial Concepts and Support Systems

What is diabulimia?

Answer

Diabulimia is the deliberate omission or restriction of insulin for weight loss in a person with type 1 diabetes. It is associated with recurrent DKA, accelerated microvascular complications, and life-threatening outcomes. Case managers address it by connecting patients with specialized eating disorder treatment that includes diabetes expertise.

#216 · Psychosocial Concepts and Support Systems

What are the warning signs of elder abuse?

Answer

Warning signs of elder abuse include unexplained injuries, weight loss and signs of neglect, fearfulness around specific caregivers, sudden changes in financial situation, and inconsistent explanations for injuries. Case managers are mandatory reporters for suspected elder abuse and must report to Adult Protective Services.

#217 · Psychosocial Concepts and Support Systems

What is financial exploitation of older adults?

Answer

Financial exploitation is the most common form of elder abuse and involves the illegal or improper use of an older adult's funds, property, or assets. Signs include sudden changes in banking activity, new signatories on accounts, and unpaid bills despite adequate income. Case managers assess and report financial exploitation.

#218 · Psychosocial Concepts and Support Systems

What is the role of faith and spirituality in patient care?

Answer

Spiritual beliefs influence how patients cope with illness, make medical decisions, and find meaning in suffering. Case managers conduct brief spiritual assessments, respect the patient's spiritual framework without imposing their own, and coordinate chaplaincy services when spiritual distress is affecting the patient's wellbeing.

#219 · Psychosocial Concepts and Support Systems

What is caregiver role conflict?

Answer

Caregiver role conflict occurs when the demands of caring for a family member directly interfere with an individual's own health self-care behaviors. It commonly presents when a patient prioritizes another person's needs over their own chronic disease management, resulting in medication non-adherence and missed appointments.

#220 · Psychosocial Concepts and Support Systems

What is illness intrusiveness?

Answer

Illness intrusiveness describes the degree to which a chronic illness and its treatment intrude into a patient's valued activities, relationships, and sense of identity. It helps explain why patients with the same diagnosis have very different quality of life experiences depending on how the illness intersects with what matters most to them.

#221 · Psychosocial Concepts and Support Systems

What is anticipatory grief?

Answer

Anticipatory grief is the emotional preparation for a future loss, experienced by patients facing terminal illness and their families before the actual death. Case managers recognize anticipatory grief, provide support, and facilitate advance care planning conversations during this period of emotional preparation.

#222 · Psychosocial Concepts and Support Systems

What is the role of the case manager in suicidal ideation?

Answer

When a patient expresses suicidal ideation, the case manager must immediately conduct a safety assessment including asking directly about plan, means, and intent, follow the organization's suicide risk protocol, and connect the patient to mental health services. Warning signs such as giving away belongings require urgent action.

#223 · Psychosocial Concepts and Support Systems

What is the Columbia Suicide Severity Rating Scale (C-SSRS)?

Answer

The C-SSRS is a validated tool for assessing suicidal ideation and behavior across a spectrum from passive ideation to specific plan with means and intent. It is widely used in healthcare settings to guide the level of intervention required and provides a structured clinical language for communicating suicide risk.

#224 · Psychosocial Concepts and Support Systems

What is the role of social work in case management?

Answer

Social work case managers specialize in psychosocial assessment, therapeutic counseling, crisis intervention, advocacy, and community resource coordination. They are particularly skilled in addressing the social determinants of health, family systems dynamics, and complex psychosocial barriers to care adherence.

#225 · Psychosocial Concepts and Support Systems

What is cognitive behavioral therapy (CBT)?

Answer

CBT is an evidence-based psychotherapy that helps patients identify and change unhelpful thought patterns and behaviors that contribute to emotional distress and poor health behaviors. It is effective for depression, anxiety, chronic pain, and health behavior change. Case managers may refer patients to CBT-trained therapists.

#226 · Psychosocial Concepts and Support Systems

What is the impact of adverse childhood experiences (ACEs) on health?

Answer

ACEs are traumatic experiences in childhood including abuse, neglect, and household dysfunction. Higher ACE scores are associated with significantly higher risks of chronic disease, mental health disorders, substance use, and premature death in adulthood. Case managers use an ACE-informed perspective to understand patients' health trajectories.

#227 · Psychosocial Concepts and Support Systems

What is substance use disorder (SUD)?

Answer

Substance use disorder is a chronic brain disorder characterized by compulsive substance use despite harmful consequences. It has a neurobiological basis and responds to evidence-based treatment including medication-assisted treatment and behavioral therapies. Case managers approach SUD as a treatable health condition, not a moral failing.

#228 · Psychosocial Concepts and Support Systems

What is the impact of social isolation on health outcomes?

Answer

Social isolation is associated with increased risk of depression, cognitive decline, cardiovascular disease, and premature mortality, comparable in magnitude to the health risks of smoking. Case managers assess social connectedness and connect isolated patients with peer support, senior programs, and community activities.

#229 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting LGBTQ+ patients?

Answer

LGBTQ+ patients face higher rates of discrimination in healthcare, mental health challenges, and social stressors that affect health. Case managers provide affirming, non-judgmental care, use correct names and pronouns, ensure providers communicate respectfully, and connect patients with LGBTQ+-affirming community resources.

#230 · Psychosocial Concepts and Support Systems

What is a bereavement program?

Answer

Bereavement programs provide emotional support, counseling, and community resources to families after the death of a patient. Medicare Hospice Benefit requires hospice agencies to provide bereavement services to families for at least one year after the patient's death. Case managers may facilitate referrals to bereavement groups.

#231 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting patients with physical disabilities?

Answer

Case managers working with patients with physical disabilities coordinate adaptive equipment, home modifications, personal care assistance, transportation, vocational rehabilitation, and community integration services. The goal is maximum independence in the least restrictive environment consistent with the patient's goals and abilities.

#232 · Psychosocial Concepts and Support Systems

What is the role of the case manager in supporting patients with intellectual and developmental disabilities?

Answer

Case managers supporting individuals with IDD coordinate medical care, behavioral health services, residential supports, employment services, and community inclusion, often working closely with the individual's support network. Understanding capacity, communication needs, and person-centered planning principles is essential.

#233 · Psychosocial Concepts and Support Systems

What is the role of cultural background in health decision-making?

Answer

Cultural background shapes health beliefs, illness explanations, acceptable treatments, the role of family in decision-making, and communication preferences. Case managers practice cultural humility by exploring each patient's specific cultural context rather than applying generalizations, and adapt their approach accordingly.

#234 · Psychosocial Concepts and Support Systems

What is an explanatory model?

Answer

An explanatory model is the patient's own understanding of what is causing their illness, why it is happening, and what would help. Understanding the patient's explanatory model is essential for developing a care plan the patient will follow, because care plans that conflict with the patient's beliefs will not be implemented.

#235 · Psychosocial Concepts and Support Systems

What is the role of case management in domestic violence?

Answer

The case manager assesses safety, provides information about domestic violence resources including hotlines and shelter, and supports the patient's autonomy in deciding how to proceed. Unlike child abuse, adult domestic violence reporting is not universally mandated; the patient's safety and autonomy guide the intervention.

#236 · Psychosocial Concepts and Support Systems

What is the concept of self-determination in case management?

Answer

Self-determination is the right of individuals to make choices about their own lives, including healthcare decisions. Case managers support self-determination by ensuring patients have the information they need to make informed choices and by respecting those choices even when the case manager disagrees with them.

#237 · Psychosocial Concepts and Support Systems

What is a brief intervention for alcohol or substance use?

Answer

A brief intervention is a short, focused conversation designed to motivate a patient to reduce harmful substance use. It typically includes asking about use, providing feedback, advising change, and assessing readiness. Brief interventions by primary care providers and case managers have demonstrated effectiveness in reducing hazardous drinking.

#238 · Psychosocial Concepts and Support Systems

What is the AUDIT-C?

Answer

The AUDIT-C is a validated three-item alcohol screening tool that asks about frequency of drinking, typical quantity, and frequency of heavy drinking. A positive screen warrants further assessment with the full AUDIT or motivational interview to explore alcohol's role in the patient's life and health.

#239 · Psychosocial Concepts and Support Systems

What is secondary traumatic stress in case managers?

Answer

Secondary traumatic stress occurs when case managers develop traumatic stress symptoms from indirect exposure to clients' trauma. It is an occupational hazard that can lead to compassion fatigue, burnout, and impaired clinical judgment. Self-care, supervision, peer support, and organizational supports are protective.

#240 · Psychosocial Concepts and Support Systems

What is the role of hope in illness and recovery?

Answer

Hope is a psychological resource that promotes active coping, engagement with treatment, and resilience in the face of serious illness. Case managers protect and nurture hope by emphasizing what is possible, connecting patients with others in recovery, and focusing on short-term achievable goals alongside long-term aspirations.

#241 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting patients with serious mental illness?

Answer

Case management for patients with serious mental illness (schizophrenia, bipolar disorder, schizoaffective disorder) focuses on medication adherence, crisis prevention, housing stability, social support, and community integration, using a recovery-oriented approach that supports the patient's goals and life aspirations.

#242 · Psychosocial Concepts and Support Systems

What is Assertive Community Treatment (ACT)?

Answer

ACT is an intensive, team-based community mental health model providing around-the-clock support to individuals with serious mental illness who have not responded to less intensive services. ACT teams include psychiatric, nursing, and social work professionals who deliver services in the patient's community setting.

#243 · Psychosocial Concepts and Support Systems

What is the relationship between poverty and mental health?

Answer

Poverty is both a cause and consequence of mental health conditions. Financial stress, inadequate housing, food insecurity, and lack of access to care all contribute to mental health challenges, while mental illness can impair educational and employment outcomes. Case managers address both clinical and economic determinants.

#244 · Psychosocial Concepts and Support Systems

What is the role of case management in first-episode psychosis programs?

Answer

First-episode psychosis programs provide early, intensive intervention after a first psychotic episode to minimize disability and support recovery. Case management in these programs coordinates psychiatric care, family support, education, and employment, using a recovery-oriented framework that preserves the patient's autonomy.

#245 · Psychosocial Concepts and Support Systems

What is psychoeducation?

Answer

Psychoeducation provides patients and families with information about a health condition, its symptoms, treatment options, and self-management strategies in an accessible, non-judgmental format. It reduces stigma, increases treatment adherence, and improves coping by helping patients understand what they are experiencing.

#246 · Psychosocial Concepts and Support Systems

What is the role of support groups in chronic disease management?

Answer

Support groups connect patients with others sharing similar health experiences, providing emotional validation, practical coping strategies, and social connection. They reduce isolation, normalize the illness experience, and often increase engagement with self-management. Case managers refer patients to condition-specific support groups.

#247 · Psychosocial Concepts and Support Systems

What is resilience and how does case management support it?

Answer

Resilience is the ability to adapt and recover in the face of adversity, stress, and trauma. Case managers support resilience by identifying and reinforcing the patient's existing strengths, connecting them with social support, building self-efficacy, and helping them identify meaning and purpose even in difficult circumstances.

#248 · Psychosocial Concepts and Support Systems

What is the role of case management in addressing health-related stigma?

Answer

Stigma related to mental illness, substance use, HIV, obesity, and other conditions can prevent patients from seeking care, disclosing information to providers, and following treatment recommendations. Case managers address stigma by using non-judgmental language, advocating for respectful care, and connecting patients with affirming providers.

#249 · Psychosocial Concepts and Support Systems

What is the social determinants of health screening tool?

Answer

Validated social determinants screening tools such as the PRAPARE (Protocol for Responding to and Assessing Patients' Assets, Risks, and Experiences) and the AHC (Accountable Health Communities) screening tool identify social needs including housing, food, transportation, and safety that affect health outcomes.

#250 · Psychosocial Concepts and Support Systems

What is the role of case management in refugee health?

Answer

Refugees may have experienced war, torture, displacement, and loss, creating complex trauma, PTSD, and distrust of authority. Case managers provide trauma-informed, culturally responsive care that acknowledges this context, works with interpreters, connects patients to refugee health programs, and builds trust gradually.

#251 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting patients facing job loss related to illness?

Answer

When illness affects employment, case managers assess the impact on income, insurance, and identity, explore ADA accommodation options before disability, assist with FMLA coordination, connect patients with vocational rehabilitation, and address the psychological impact of occupational loss on self-esteem and purpose.

#252 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting patients with chronic pain?

Answer

Chronic pain case management coordinates pain management, behavioral health, physical rehabilitation, and functional restoration. It addresses the psychological dimensions of pain including depression, anxiety, catastrophizing, and fear-avoidance, which significantly influence pain severity and disability.

#253 · Psychosocial Concepts and Support Systems

What is the role of end-of-life psychosocial support in case management?

Answer

End-of-life case management addresses not only comfort-focused medical care but also the patient's and family's emotional, relational, and spiritual needs during the dying process. Case managers facilitate meaning-making conversations, support family communication, and connect the patient with bereavement-preparatory resources.

#254 · Psychosocial Concepts and Support Systems

What is the role of the case manager in medication adherence?

Answer

Medication non-adherence has multiple causes including side effects, cost, complexity, denial, cultural beliefs, and forgetting. The case manager explores the specific cause for the individual patient and tailors the intervention accordingly, whether that is medication assistance programs, education, simplification, or addressing beliefs.

#255 · Psychosocial Concepts and Support Systems

What is the difference between adherence and compliance?

Answer

Compliance implies the patient follows the provider's instructions passively. Adherence acknowledges the patient's active role in making choices about their care. Current best practice uses adherence to reflect a collaborative model where the patient and provider agree on a care plan the patient is motivated and able to follow.

#256 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting older adults with depression?

Answer

Late-life depression is underdiagnosed and undertreated, often misattributed to normal aging. Case managers screen with tools like the PHQ-9 or GDS, advocate for evaluation and treatment, connect patients with mental health services or collaborative care programs, and address social isolation that contributes to depression.

#257 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting caregivers of people with dementia?

Answer

Dementia caregiving is one of the most burdensome caregiving roles due to the duration, unpredictability, and emotional toll of the role. Case managers assess caregiver burden, connect caregivers with respite care, support groups, the Alzheimer's Association, and adult day programs, and monitor for caregiver depression and physical deterioration.

#258 · Psychosocial Concepts and Support Systems

What is the ACES framework in clinical care?

Answer

The Adverse Childhood Experiences framework demonstrates that cumulative childhood trauma is strongly associated with adult chronic disease, mental health disorders, and premature mortality through biopsychosocial pathways. Case managers use an ACE-informed perspective to understand the health histories of complex patients without judgment.

#259 · Psychosocial Concepts and Support Systems

What is health coaching?

Answer

Health coaching uses evidence-based behavior change techniques to help patients develop the skills, knowledge, and confidence to make sustainable changes that support chronic disease management. It is distinct from case management (broader coordination role) and disease management (population-based protocol).

#260 · Psychosocial Concepts and Support Systems

What is the role of spirituality in end-of-life care?

Answer

Spiritual distress at end of life can manifest as existential questioning, loss of meaning, or unresolved relational conflicts. Case managers conduct brief spiritual assessments, facilitate chaplaincy referrals, and create space for patients and families to address spiritual concerns as part of holistic end-of-life care.

#261 · Psychosocial Concepts and Support Systems

What is the impact of chronic illness on family systems?

Answer

Chronic illness ripples through the entire family system, changing roles, relationships, and communication patterns. Case managers assess family dynamics, identify which members are carrying disproportionate burden, and facilitate family meetings to clarify the care plan, redistribute responsibilities, and address interpersonal conflicts.

#262 · Psychosocial Concepts and Support Systems

What is body image and its relevance to case management?

Answer

Body image concerns arise frequently after disfiguring illness, surgery, colostomy, amputation, or significant weight change. Case managers normalize these concerns, connect patients with peer support and counseling, and advocate for rehabilitation services (such as prosthetics and reconstructive care) that support body image restoration.

#263 · Psychosocial Concepts and Support Systems

What is the role of case management in addressing health anxiety?

Answer

Health anxiety (formerly hypochondria) involves persistent, excessive worry about having or developing a serious illness. Case managers provide reassurance without reinforcing avoidance, collaborate with the treatment team to establish clear communication protocols, and refer to CBT which has the strongest evidence for health anxiety.

#264 · Psychosocial Concepts and Support Systems

What is the relationship between chronic pain and depression?

Answer

Chronic pain and depression are bidirectionally linked: pain contributes to depression through functional limitation and hopelessness, while depression amplifies pain perception through shared neurobiological pathways. Effective treatment of chronic pain must address both the physical and psychological dimensions simultaneously.

#265 · Psychosocial Concepts and Support Systems

What is the role of the community mental health center?

Answer

Community mental health centers provide outpatient psychiatric services, crisis intervention, case management, peer support, and community-based rehabilitation for individuals with serious mental illness. They are a primary referral resource for case managers coordinating behavioral health services for uninsured or Medicaid-covered patients.

#266 · Psychosocial Concepts and Support Systems

What is the CAGE questionnaire?

Answer

The CAGE is a brief four-question alcohol screening tool asking about cutting down, annoyance at criticism, guilt about drinking, and eye-opener drinks. Two or more positive answers suggest alcohol use disorder and warrant further clinical evaluation. It is less sensitive than the AUDIT-C but very quick to administer.

#267 · Psychosocial Concepts and Support Systems

What is the role of case management in eating disorders?

Answer

Eating disorder case management coordinates medical stabilization (when weight is critically low or medical complications are present), psychiatric care, nutritional therapy, and family therapy. Case managers understand that eating disorders carry the highest mortality rate of any psychiatric diagnosis and treat them with appropriate urgency.

#268 · Psychosocial Concepts and Support Systems

What is the role of the case manager in supporting patients with intellectual disabilities?

Answer

Case managers supporting patients with intellectual disabilities use clear, simple communication, involve the patient as a decision-maker to the extent of their capacity, work closely with support persons who know the patient well, and ensure that healthcare providers communicate in accessible ways that respect the patient's dignity.

#269 · Psychosocial Concepts and Support Systems

What is the Geriatric Depression Scale (GDS)?

Answer

The GDS is a validated depression screening tool developed specifically for older adults, using a yes/no format that is easier for cognitively impaired or low-literacy patients than the PHQ-9. Short forms with 15 or 5 items exist. A positive screen warrants further clinical evaluation and intervention.

#270 · Psychosocial Concepts and Support Systems

What is the Mini-Mental State Examination (MMSE)?

Answer

The MMSE is a validated 30-point cognitive screening tool assessing orientation, registration, attention, recall, and language. Scores below 24 suggest possible cognitive impairment requiring further evaluation. It is used to identify patients who may need surrogate decision-making support or simplified care plan communication.

#271 · Psychosocial Concepts and Support Systems

What is the Montreal Cognitive Assessment (MoCA)?

Answer

The MoCA is a more sensitive cognitive screening tool than the MMSE, particularly for detecting mild cognitive impairment. It assesses visuospatial ability, executive function, memory, attention, language, and orientation. Scores below 26 suggest possible cognitive impairment requiring further evaluation.

#272 · Psychosocial Concepts and Support Systems

What is patient-reported outcome measures (PROMs)?

Answer

PROMs capture the patient's own perspective on their health status, symptoms, functional ability, and quality of life using validated questionnaires. They are distinct from clinician-observed or administrative outcomes and provide a direct measure of what matters most to the patient from the patient's own perspective.

#273 · Psychosocial Concepts and Support Systems

What is the role of hope in motivational interviewing?

Answer

Evoking the patient's own hope for change is a core MI strategy. The case manager reflects statements that suggest a better future is possible, explores the patient's vision of life without the problem, and reinforces change talk that expresses desire, ability, or reason for change as evidence of the patient's own hope.

#274 · Psychosocial Concepts and Support Systems

What is the role of case management in perinatal mental health?

Answer

Perinatal depression and anxiety affect up to 20% of pregnant and postpartum women and significantly affect maternal-infant attachment and child development. Case managers screen with validated tools, facilitate urgent referral for positive screens, and coordinate between obstetric, psychiatric, and primary care providers.

#275 · Psychosocial Concepts and Support Systems

What is the role of social prescribing in case management?

Answer

Social prescribing connects patients with community activities, groups, and services to address social, emotional, and practical needs that cannot be met through clinical care alone. Examples include referrals to art classes, gardening groups, volunteering, and befriending programs for socially isolated patients.

#276 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting chronic homelessness?

Answer

Chronically homeless patients have significantly higher rates of mental illness, substance use, chronic disease, and mortality. Case managers connect them with Housing First programs, assertive case management, mental health treatment, substance use services, and medical respite programs that provide a stable environment for recovery.

#277 · Psychosocial Concepts and Support Systems

What is Housing First?

Answer

Housing First is an evidence-based approach to ending homelessness that prioritizes immediate permanent housing without requiring sobriety or treatment compliance as a precondition. Research consistently shows that stable housing is the necessary foundation for addressing mental health and substance use challenges.

#278 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting patients with HIV-related stigma?

Answer

HIV-related stigma can prevent patients from disclosing status to partners, seeking care, or adhering to treatment. Case managers provide affirming care, connect patients with peer support, advocate for respectful treatment from the healthcare team, and address the mental health impact of stigma on the patient's wellbeing.

#279 · Psychosocial Concepts and Support Systems

What is the concept of illness identity?

Answer

Illness identity refers to the degree to which a patient defines themselves by their illness or integrates it as one part of a larger self-concept. Patients who over-identify with their illness may struggle with behavior change that threatens this identity. Case managers support a balanced illness identity that acknowledges the condition without reducing the person to it.

#280 · Psychosocial Concepts and Support Systems

What is the Distress Thermometer?

Answer

The Distress Thermometer is a validated single-item screening tool used in oncology settings. Patients rate their distress from 0 to 10, and those scoring 4 or above are referred for further assessment. A companion problem list identifies the specific contributors to distress including practical, family, emotional, and physical concerns.

#281 · Psychosocial Concepts and Support Systems

What is the role of case management in promoting mental health in the general population?

Answer

Beyond clinical populations, case managers in population health settings address mental wellness through screening, education, stigma reduction, and connection to community resources. Mental health promotion activities delay the onset of disorders and reduce the burden of untreated mental illness on healthcare systems.

#282 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting patients after natural disaster?

Answer

Following disasters, case managers assess patients' acute safety, housing, medication access, and mental health, prioritizing those with chronic conditions who have lost access to care and medications. Mental health response includes grief support, PTSD screening, and connection to disaster relief mental health services.

#283 · Psychosocial Concepts and Support Systems

What is vicarious trauma?

Answer

Vicarious trauma is the cumulative transformation in the case manager's worldview resulting from empathic engagement with traumatized patients over time. Unlike secondary traumatic stress (acute stress reaction), vicarious trauma is a gradual shift in beliefs about safety, trust, and meaning. Supervision, peer support, and self-care strategies are protective.

#284 · Psychosocial Concepts and Support Systems

What is compassion satisfaction?

Answer

Compassion satisfaction is the pleasure and fulfillment that case managers derive from doing their work well, helping patients, and contributing to positive outcomes. Building awareness of compassion satisfaction as a counterbalance to compassion fatigue supports professional resilience and sustainability in the case management role.

#285 · Psychosocial Concepts and Support Systems

What is the role of case management in supporting patients in recovery from addiction?

Answer

Recovery-oriented case management for addiction supports the patient's long-term journey through treatment, relapse, and ongoing recovery. It connects patients with peer support, recovery community organizations, stable housing, employment assistance, and ongoing medication management, recognizing relapse as a common part of the recovery process.

#286 · Quality and Outcomes Evaluation and Measurements

What are the six aims of the IOM's 'Crossing the Quality Chasm'?

Answer

The IOM's Crossing the Quality Chasm (2001) identified six aims for a high-quality healthcare system: safe, effective, patient-centered, timely, efficient, and equitable. These aims provide the framework for quality measurement and improvement programs across healthcare settings.

#287 · Quality and Outcomes Evaluation and Measurements

What is the Triple Aim?

Answer

The Triple Aim, developed by the Institute for Healthcare Improvement, describes three simultaneous goals: improving the patient experience of care, improving the health of populations, and reducing the per-capita cost of care. These goals must be pursued together rather than in isolation.

#288 · Quality and Outcomes Evaluation and Measurements

What is the Quadruple Aim?

Answer

The Quadruple Aim adds a fourth goal to the Triple Aim: improving the work life of healthcare providers. Clinician burnout is recognized as a quality and safety problem because it leads to errors and workforce attrition. Case managers who reduce unnecessary work and improve communication contribute to provider well-being.

#289 · Quality and Outcomes Evaluation and Measurements

What is the PDCA/PDSA cycle?

Answer

The PDCA (Plan-Do-Check-Act) or PDSA (Plan-Do-Study-Act) cycle is the foundational quality improvement model. It involves planning a change, implementing it on a small scale, collecting data to evaluate the result, and then deciding whether to adopt, adapt, or abandon the change before the next cycle.

#290 · Quality and Outcomes Evaluation and Measurements

What is Lean in healthcare quality improvement?

Answer

Lean is a quality improvement methodology focused on identifying and eliminating waste (unnecessary steps, waiting, defects, overproduction) in healthcare processes. It originates from the Toyota Production System and uses tools like value stream mapping, 5S, and rapid improvement Kaizen events.

#291 · Quality and Outcomes Evaluation and Measurements

What is Six Sigma in healthcare?

Answer

Six Sigma is a data-driven quality improvement methodology that uses the DMAIC process (Define, Measure, Analyze, Improve, Control) to reduce variation and bring defect rates toward near-zero. It uses statistical tools to identify root causes and implement solutions that are monitored for sustained performance.

#292 · Quality and Outcomes Evaluation and Measurements

What is DMAIC?

Answer

DMAIC stands for Define, Measure, Analyze, Improve, and Control. It is the structured five-phase process used in Six Sigma projects to systematically reduce variation and improve process quality. Each phase has specific tools and deliverables that ensure the improvement is data-driven and sustainable.

#293 · Quality and Outcomes Evaluation and Measurements

What is root cause analysis (RCA)?

Answer

RCA is a structured investigation of a serious adverse event or near-miss that seeks to identify the underlying systemic causes rather than blaming individuals. RCA uses tools such as the 5 Whys, fishbone (Ishikawa) diagrams, and process mapping, and results in recommendations for systemic change.

#294 · Quality and Outcomes Evaluation and Measurements

What is a fishbone (Ishikawa) diagram?

Answer

A fishbone diagram is a root cause analysis tool that visually organizes potential causes of a problem by category (often called the 6 M's: Man, Machine, Method, Material, Measurement, Mother Nature). It helps teams brainstorm comprehensively and identify the most likely root causes for investigation.

#295 · Quality and Outcomes Evaluation and Measurements

What is the 5 Whys technique?

Answer

The 5 Whys is a root cause analysis technique that repeatedly asks 'why' to trace a problem from its surface symptom to its underlying systemic cause. By asking why five or more times, teams move past obvious immediate causes to identify the root cause that, when addressed, will prevent recurrence.

#296 · Quality and Outcomes Evaluation and Measurements

What is a process measure in quality?

Answer

A process measure tracks whether providers are performing evidence-based actions that are known to improve outcomes. For example, the percentage of heart failure patients who received discharge instructions or the percentage of diabetic patients who received an annual A1c test are process measures.

#297 · Quality and Outcomes Evaluation and Measurements

What is an outcome measure in quality?

Answer

An outcome measure tracks the results of care such as 30-day readmission rates, mortality rates, hemoglobin A1c levels in diabetic patients, or blood pressure control rates. Outcome measures directly reflect what happened to patients and are the most meaningful indicators of care quality.

#298 · Quality and Outcomes Evaluation and Measurements

What is a structural measure in quality?

Answer

A structural measure assesses characteristics of the care environment such as staffing ratios, electronic health record capabilities, provider credentials, and facility accreditation. It reflects the capacity of the organization to provide high-quality care rather than the processes or outcomes of care itself.

#299 · Quality and Outcomes Evaluation and Measurements

What is a patient experience measure?

Answer

A patient experience measure captures how patients perceive their care, including communication quality, coordination, respect, and responsiveness. HCAHPS is the primary patient experience measure for hospitals. Patient experience is distinct from patient satisfaction, though the terms are often used interchangeably.

#300 · Quality and Outcomes Evaluation and Measurements

What is HCAHPS?

Answer

HCAHPS (Hospital Consumer Assessment of Healthcare Providers and Systems) is a standardized patient experience survey used to measure patient perception of hospital care. CMS publicly reports HCAHPS scores and uses them in the Hospital Value-Based Purchasing program, linking them to hospital payment.

#301 · Quality and Outcomes Evaluation and Measurements

What is HEDIS?

Answer

HEDIS (Healthcare Effectiveness Data and Information Set) is a set of quality measures used by more than 90% of U.S. health plans to measure performance. HEDIS measures cover preventive care, chronic disease management, behavioral health, and patient safety, and are used for NCQA accreditation and public reporting.

#302 · Quality and Outcomes Evaluation and Measurements

What are Core Measures?

Answer

Core Measures are evidence-based quality measures developed by the Joint Commission and CMS for hospital quality reporting. They include condition-specific measure sets for acute MI, heart failure, pneumonia, stroke, and surgical care. Performance on Core Measures is publicly reported and affects hospital reimbursement.

#303 · Quality and Outcomes Evaluation and Measurements

What is the Joint Commission and what does it accredit?

Answer

The Joint Commission is the largest accrediting body for healthcare organizations in the U.S. It accredits hospitals, nursing homes, home health agencies, behavioral health programs, and other organizations. Joint Commission accreditation confers deemed status for Medicare and Medicaid participation.

#304 · Quality and Outcomes Evaluation and Measurements

What is NCQA and what does it accredit?

Answer

NCQA (National Committee for Quality Assurance) accredits health plans and medical groups and provides PCMH recognition. NCQA manages HEDIS, the most widely used managed care quality measure set. NCQA accreditation is important for health plans seeking contracts with large employers and government programs.

#305 · Quality and Outcomes Evaluation and Measurements

What is URAC?

Answer

URAC (formerly Utilization Review Accreditation Commission) accredits case management programs, disease management programs, and health plans. URAC accreditation signals that a case management program meets rigorous standards for operations, quality, and consumer protection. It is the most relevant accreditor for case management programs specifically.

#306 · Quality and Outcomes Evaluation and Measurements

What is a Never Event?

Answer

A Never Event is a serious, largely preventable adverse event that should never occur in a healthcare setting. CMS designates a list of Hospital-Acquired Conditions for which it will not provide additional reimbursement, including wrong-site surgery, stage 3 and 4 pressure injuries, and certain falls with injury.

#307 · Quality and Outcomes Evaluation and Measurements

What is a sentinel event?

Answer

A sentinel event is any unexpected event involving death or serious physical or psychological harm not related to the natural course of illness. The Joint Commission requires accredited organizations to conduct a root cause analysis after a sentinel event and implement a plan to prevent recurrence.

#308 · Quality and Outcomes Evaluation and Measurements

What is the difference between a Never Event and a sentinel event?

Answer

A Never Event is a CMS-defined category of preventable adverse events for which additional reimbursement is denied. A sentinel event is the Joint Commission's broader category of serious unexpected adverse events that trigger required RCA. All Never Events are serious events but not all are Joint Commission sentinel events and vice versa.

#309 · Quality and Outcomes Evaluation and Measurements

What is benchmarking?

Answer

Benchmarking compares an organization's performance on quality measures against an external standard, which may be a national average, a top-decile performance level, or a peer group. Benchmarking identifies performance gaps and motivates improvement by showing what is achievable.

#310 · Quality and Outcomes Evaluation and Measurements

What is a run chart?

Answer

A run chart plots a quality measure over time and allows visual identification of trends, shifts, and unusual patterns. Unlike a control chart, it does not have statistical control limits, but it provides a simple visual tool for tracking whether improvement efforts are producing change over time.

#311 · Quality and Outcomes Evaluation and Measurements

What is a control chart?

Answer

A control chart adds upper and lower statistical control limits to time-series data, allowing distinction between normal (common cause) variation and signals (special cause variation) that indicate real change in the process. It is a more rigorous tool than a run chart for monitoring quality improvement progress.

#312 · Quality and Outcomes Evaluation and Measurements

What is the Braden Scale?

Answer

The Braden Scale is the most widely used validated tool for assessing a patient's risk of developing pressure injuries. It scores six risk factors: sensory perception, moisture, activity, mobility, nutrition, and friction/shear. Lower scores indicate higher risk and trigger preventive interventions.

#313 · Quality and Outcomes Evaluation and Measurements

What is the Morse Fall Scale?

Answer

The Morse Fall Scale is a validated tool for assessing hospital patient fall risk. It scores six factors including history of falling, secondary diagnosis, ambulatory aid use, IV line status, gait, and mental status. Patients scoring above 45 are considered high risk and receive fall prevention interventions.

#314 · Quality and Outcomes Evaluation and Measurements

What is evidence-based practice (EBP)?

Answer

EBP integrates the best available research evidence, clinical expertise, and patient preferences and values to guide clinical decisions. It represents the highest standard of professional practice and is distinguished from reliance on tradition, authority, or anecdote.

#315 · Quality and Outcomes Evaluation and Measurements

What is the hierarchy of evidence?

Answer

The hierarchy of evidence ranks research designs by strength: systematic reviews and meta-analyses at the top, followed by randomized controlled trials, cohort studies, case-control studies, case reports, and expert opinion at the bottom. Case managers use the highest available evidence level to guide practice decisions.

#316 · Quality and Outcomes Evaluation and Measurements

What is a systematic review and meta-analysis?

Answer

A systematic review synthesizes all published studies on a specific clinical question using a rigorous, replicable methodology. A meta-analysis combines data from multiple studies statistically to produce a pooled estimate of effect. Together, they represent the highest level of evidence in the hierarchy.

#317 · Quality and Outcomes Evaluation and Measurements

What is a randomized controlled trial (RCT)?

Answer

An RCT is the gold standard for testing the effectiveness of clinical interventions. Participants are randomly assigned to treatment or control groups, minimizing selection bias and allowing causal inference about the effect of the intervention. Case managers use RCT evidence to evaluate the interventions they recommend.

#318 · Quality and Outcomes Evaluation and Measurements

What is variance analysis in case management?

Answer

Variance analysis examines differences between the expected and actual outcomes for a patient or episode of care. When outcomes deviate from the expected clinical pathway, the case manager identifies the reason for the variance (patient, clinician, system, or community) and intervenes to address correctable factors.

#319 · Quality and Outcomes Evaluation and Measurements

What is predictive analytics in case management?

Answer

Predictive analytics use historical clinical, claims, and social data to estimate which patients are at highest risk for hospitalization, readmission, or clinical deterioration. Case managers use predictive models to identify high-risk patients proactively and target intensive interventions before a crisis occurs.

#320 · Quality and Outcomes Evaluation and Measurements

What is the CAHPS Case Management Survey?

Answer

The CAHPS Case Management Survey is a validated patient experience survey designed specifically for case management programs. It measures how patients perceive the quality of their case management interaction including communication clarity, responsiveness, and impact on their ability to manage their health.

#321 · Quality and Outcomes Evaluation and Measurements

What is the difference between quality assurance and quality improvement?

Answer

Quality assurance (QA) is a retrospective, inspection-based approach that identifies and corrects deficiencies after they occur. Quality improvement (QI) is a proactive, systems-based approach that uses data to continuously improve processes. Modern healthcare emphasizes QI over QA as a more effective and sustainable model.

#322 · Quality and Outcomes Evaluation and Measurements

What is a reportable event in quality management?

Answer

Reportable events are incidents that must be reported to external bodies. States require reporting of sentinel events to state health departments. CMS requires reporting of Hospital-Acquired Conditions. Suspected abuse and neglect must be reported to state protective service agencies. Case managers must know their mandatory reporting obligations.

#323 · Quality and Outcomes Evaluation and Measurements

What are National Patient Safety Goals (NPSGs)?

Answer

NPSGs are the Joint Commission's annual evidence-based safety requirements for accredited organizations. Key NPSGs include using two patient identifiers, improving staff communication, using medicines safely, preventing infections, identifying patient safety risks (including suicide), and preventing surgical errors.

#324 · Quality and Outcomes Evaluation and Measurements

What is medication safety and high-alert medications?

Answer

High-alert medications are drugs that bear heightened risk of significant patient harm when used in error, as identified by the Institute for Safe Medication Practices (ISMP). High-alert medications include anticoagulants, insulin, concentrated electrolytes, and opioids, which require special safeguards and double-checks.

#325 · Quality and Outcomes Evaluation and Measurements

What is the role of case management in quality improvement?

Answer

Case managers contribute to quality improvement by tracking patient outcomes, identifying patterns in readmissions and gaps in care, participating in RCA and QI committees, implementing evidence-based protocols, and generating data that demonstrates the impact of case management programs on population health.

#326 · Quality and Outcomes Evaluation and Measurements

What is a quality dashboard?

Answer

A quality dashboard is a visual display of key performance indicators that allows leadership and care teams to monitor quality metrics at a glance, identify trends, and track progress toward improvement goals. Case management programs often track readmission rates, patient satisfaction, and clinical outcome measures on dashboards.

#327 · Quality and Outcomes Evaluation and Measurements

What is a case management program outcome report?

Answer

A case management outcome report tracks the clinical, cost, functional, and patient experience results achieved by the program. It demonstrates the program's value to leadership and payers by quantifying improvements in hospitalizations, readmissions, disease control, and cost of care in the enrolled population.

#328 · Quality and Outcomes Evaluation and Measurements

What is continuous quality improvement (CQI)?

Answer

CQI is an ongoing organizational commitment to using data and structured improvement methods to make processes more effective and efficient over time. It is characterized by small, incremental cycles of improvement rather than one-time fixes, and involves frontline staff in identifying and testing changes.

#329 · Quality and Outcomes Evaluation and Measurements

What is a quality improvement team?

Answer

A QI team is a cross-functional group of people who work together on a specific improvement project, typically including clinical staff, administrative staff, and patients or family members. Diverse perspectives improve the quality of problem analysis and the feasibility of proposed solutions.

#330 · Quality and Outcomes Evaluation and Measurements

What is the role of patient and family engagement in quality improvement?

Answer

Patients and family members bring unique perspectives on where care processes fail and what matters most in the care experience. Their involvement in QI teams, advisory councils, and co-design processes leads to more effective and acceptable improvement interventions than clinician-only design.

#331 · Quality and Outcomes Evaluation and Measurements

What is a clinical pathway?

Answer

A clinical pathway (also called a care pathway or critical pathway) is a standardized, evidence-based plan for managing a specific condition or procedure, outlining expected interventions, outcomes, and timelines. Variance from the pathway is tracked to identify opportunities for improvement.

#332 · Quality and Outcomes Evaluation and Measurements

What is fall prevention in hospital settings?

Answer

Fall prevention programs include admission fall risk assessment using validated tools, hourly rounding, bed alarms, non-slip footwear, call light accessibility, medication review for fall-risk drugs, and patient education. Falls are the leading cause of injury in hospitalized patients and are classified as preventable Never Events when they result in serious injury.

#333 · Quality and Outcomes Evaluation and Measurements

What is a pressure injury and how is it classified?

Answer

Pressure injuries result from sustained pressure reducing blood flow to skin and underlying tissue. Stages I-IV describe increasing depth of tissue involvement; unstageable injuries have tissue obscured by necrosis; deep tissue injuries show intact skin with deep tissue damage. Stage 3 and 4 hospital-acquired pressure injuries are HACs for which CMS withholds additional payment.

#334 · Quality and Outcomes Evaluation and Measurements

What is a catheter-associated urinary tract infection (CAUTI)?

Answer

CAUTI is a common healthcare-associated infection caused by bacteria entering through an indwelling urinary catheter. It is a designated HAC for which CMS does not provide additional payment. Prevention bundles include appropriate catheter insertion technique, daily assessment of catheter necessity, and prompt removal when no longer needed.

#335 · Quality and Outcomes Evaluation and Measurements

What is a central line-associated bloodstream infection (CLABSI)?

Answer

CLABSI is a serious, largely preventable bloodstream infection associated with a central venous catheter. Prevention bundles include hand hygiene, maximal barrier precautions, chlorhexidine skin antisepsis, optimal site selection, daily assessment of line necessity, and prompt removal. CLABSI rates are publicly reported quality measures.

#336 · Quality and Outcomes Evaluation and Measurements

What is a ventilator-associated pneumonia (VAP)?

Answer

VAP is pneumonia developing in patients on mechanical ventilation 48 hours or more after intubation. Prevention bundles include head-of-bed elevation to 30-45 degrees, daily sedation vacation and readiness-to-extubate assessment, oral care with chlorhexidine, and deep vein thrombosis and peptic ulcer prophylaxis.

#337 · Quality and Outcomes Evaluation and Measurements

What is hand hygiene compliance and why does it matter?

Answer

Hand hygiene is the single most effective intervention to prevent healthcare-associated infections. Compliance rates in healthcare settings are historically low despite universal knowledge of its importance. Monitoring hand hygiene compliance and providing real-time feedback are core safety program components.

#338 · Quality and Outcomes Evaluation and Measurements

What is total cost of care?

Answer

Total cost of care is the complete cost of all healthcare services used by a defined patient population over a defined period, including inpatient, outpatient, pharmacy, post-acute, and behavioral health costs. It is the primary financial outcome measure in value-based contracts and ACO programs.

#339 · Quality and Outcomes Evaluation and Measurements

What is avoidable utilization?

Answer

Avoidable utilization refers to healthcare services that could have been prevented with appropriate care management, including preventable hospitalizations, avoidable ED visits, and readmissions caused by inadequate discharge preparation. Reducing avoidable utilization is a primary measure of case management program effectiveness.

#340 · Quality and Outcomes Evaluation and Measurements

What is a readmission and how is it measured?

Answer

A readmission is a return to the hospital within a defined time period, most commonly 30 days, after discharge. CMS tracks condition-specific 30-day readmission rates for conditions including heart failure, AMI, pneumonia, hip and knee replacement, COPD, and stroke, with payment penalties for hospitals with excess readmissions.

#341 · Quality and Outcomes Evaluation and Measurements

What is patient safety culture?

Answer

Patient safety culture is the shared values, beliefs, and behaviors within a healthcare organization that support safe care. Key components include psychological safety to report errors without fear, systems thinking rather than individual blame, leadership commitment to safety, and learning from near-misses and adverse events.

#342 · Quality and Outcomes Evaluation and Measurements

What is just culture in patient safety?

Answer

Just culture distinguishes between human error (system-caused, non-punishable), at-risk behavior (habit that increases risk, addressed through coaching), and reckless behavior (knowingly ignoring safety rules, disciplined). It creates an environment where staff report errors and near-misses without fear of unjust punishment.

#343 · Quality and Outcomes Evaluation and Measurements

What is failure mode and effects analysis (FMEA)?

Answer

FMEA is a proactive risk assessment tool that systematically identifies potential failure points in a process before they cause harm. Teams rank failures by severity, likelihood of occurrence, and detectability to prioritize which risks to address first. It is used preventively rather than reactively like RCA.

#344 · Quality and Outcomes Evaluation and Measurements

What is the Hospital Consumer Assessment of Healthcare Providers and Systems (HCAHPS) survey structure?

Answer

HCAHPS surveys cover eight core domains: communication with nurses, communication with doctors, responsiveness of staff, communication about medications, discharge information, care transitions, hospital environment, and overall hospital rating. CMS publicly reports HCAHPS scores and uses them in hospital value-based purchasing payment calculations.

#345 · Quality and Outcomes Evaluation and Measurements

What is person-centered quality measurement?

Answer

Person-centered quality measurement captures outcomes that matter most to patients, including functional status, symptom burden, quality of life, and goal attainment, rather than focusing exclusively on clinical or financial metrics. Patient-reported outcomes measures (PROMs) are the primary tool for person-centered quality measurement.

#346 · Quality and Outcomes Evaluation and Measurements

What is a health technology assessment (HTA)?

Answer

HTA systematically evaluates the clinical effectiveness, safety, and cost-effectiveness of health technologies including drugs, devices, and procedures. HTA findings are used by payers to make coverage decisions and by health systems to guide formulary and purchasing decisions.

#347 · Quality and Outcomes Evaluation and Measurements

What is a care gap?

Answer

A care gap is a documented clinical need that has not been addressed, such as a diabetic patient who has not received an annual A1c test or a patient on anticoagulation without INR monitoring. Identifying and closing care gaps is a core function of population health management programs and health plan quality programs.

#348 · Quality and Outcomes Evaluation and Measurements

What is the role of the electronic health record (EHR) in quality improvement?

Answer

EHRs enable quality improvement by providing clinical decision support alerts, facilitating population-level reporting, enabling care gap identification, supporting care coordination across settings, and generating quality measure data. EHR-generated quality data is increasingly used for public reporting and value-based payment programs.

#349 · Quality and Outcomes Evaluation and Measurements

What is a clinical decision support (CDS) system?

Answer

CDS systems provide clinicians with patient-specific information and clinical knowledge at the point of care to enhance decision-making. Examples include drug interaction alerts, order sets aligned with clinical guidelines, and reminders for preventive services. CDS can reduce errors and improve adherence to evidence-based guidelines.

#350 · Quality and Outcomes Evaluation and Measurements

What is accountable care in quality terms?

Answer

Accountable care means that providers and health systems take responsibility for the quality and total cost of care for a defined population, with financial consequences for performance above or below benchmarks. It shifts the quality measurement focus from individual visits to longitudinal population health outcomes.

#351 · Quality and Outcomes Evaluation and Measurements

What are quality-adjusted life years (QALYs)?

Answer

A QALY combines the quantity of life (years of survival) with the quality of life (health state utilities on a 0-1 scale) into a single metric. Cost-effectiveness analyses use QALYs to compare the value of different interventions. One QALY gained is often considered cost-effective below \$50,000-\$100,000 in U.S. healthcare.

#352 · Quality and Outcomes Evaluation and Measurements

What is the role of case management in preventing hospital-acquired conditions?

Answer

Case managers identify patients at high risk for HACs through systematic screening on admission, ensure that evidence-based prevention protocols are activated, monitor compliance with prevention bundles, escalate concerns when protocols are not followed, and track HAC rates as quality outcomes for the case management program.

#353 · Quality and Outcomes Evaluation and Measurements

What is a care transition program?

Answer

Care transition programs provide structured support during the handoff from hospital to home or post-acute care to reduce preventable readmissions. Evidence-based models include the Care Transitions Intervention (Coleman Model) and Project RED, which include patient education, medication reconciliation, and post-discharge follow-up.

#354 · Quality and Outcomes Evaluation and Measurements

What is the Coleman Care Transitions Intervention?

Answer

The Coleman Care Transitions Intervention is a four-week evidence-based program in which a Transitions Coach (typically an advanced practice nurse) visits the patient in the hospital, accompanies them home, and makes follow-up calls to build self-management skills and ensure care continuity. It has demonstrated significant reductions in 30-day and 90-day readmissions.

#355 · Quality and Outcomes Evaluation and Measurements

What is Project RED (Re-Engineered Discharge)?

Answer

Project RED is an evidence-based hospital discharge process that includes 11 components: after-visit summary, medication reconciliation, follow-up appointments, patient education, and a post-discharge phone call. Implementation of Project RED has been shown to significantly reduce 30-day readmission rates.

#356 · Quality and Outcomes Evaluation and Measurements

What is the BOOST program (Better Outcomes for Older Adults through Safe Transitions)?

Answer

BOOST is a Society of Hospital Medicine program providing tools and mentoring to help hospitals improve discharge processes and reduce readmissions for older adults. BOOST tools include the 8P risk assessment identifying discharge risk factors and an evidence-based discharge bundle addressing those risks.

#357 · Quality and Outcomes Evaluation and Measurements

What is case mix index (CMI)?

Answer

CMI is the average DRG weight for all Medicare patients in a hospital, reflecting the clinical complexity and resource intensity of the patient population served. A higher CMI indicates a more complex patient mix and generally correlates with higher reimbursement per case. Case management documentation quality affects CMI accuracy.

#358 · Quality and Outcomes Evaluation and Measurements

What is clinical documentation improvement (CDI)?

Answer

CDI is the practice of improving clinical documentation to accurately reflect the complexity and severity of patient illness, ensuring that coded diagnoses and procedures correctly represent the care provided. Accurate CDI supports appropriate DRG assignment, quality reporting, and reimbursement.

#359 · Quality and Outcomes Evaluation and Measurements

What is the role of case management in population health analytics?

Answer

Case management generates data that supports population health analytics including utilization patterns, care gap prevalence, social determinants distribution, and intervention outcomes. This data is used to refine risk stratification models, evaluate program effectiveness, and identify emerging quality concerns.

#360 · Quality and Outcomes Evaluation and Measurements

What is health information exchange (HIE)?

Answer

HIE is the electronic sharing of clinical information across healthcare organizations to support coordinated care. It allows case managers and providers to access a patient's complete medication list, recent labs, and prior hospitalizations across organizations. HIE improves care transitions and reduces duplicative testing.

#361 · Quality and Outcomes Evaluation and Measurements

What is Lean's concept of 'muda' (waste)?

Answer

In Lean healthcare, waste (muda) refers to any activity that consumes resources without adding value for the patient. The eight wastes include overproduction, waiting, unnecessary transportation, over-processing, excess inventory, unnecessary motion, defects, and underutilization of talent. Lean tools identify and eliminate these wastes.

#362 · Quality and Outcomes Evaluation and Measurements

What is a Kaizen event?

Answer

A Kaizen event (also called a rapid improvement event) is an intensive, focused team effort to make rapid improvements to a specific process. Over three to five days, a multidisciplinary team maps the current process, identifies waste and problems, designs a future state, and implements changes. Results are measured against pre-event baselines.

#363 · Quality and Outcomes Evaluation and Measurements

What is value stream mapping?

Answer

Value stream mapping is a Lean tool that diagrams all steps in a care process, identifying which steps add value from the patient's perspective and which are waste. It makes the entire flow of care visible, creating a shared understanding of where delays, redundancies, and inefficiencies occur.

#364 · Quality and Outcomes Evaluation and Measurements

What is a clinical registry?

Answer

A clinical registry is a database that systematically collects standardized data on patients with a specific condition, procedure, or characteristic. Registries support quality monitoring, outcomes research, and identification of care gaps. They are used in value-based care to track population health metrics for enrolled patients.

#365 · Quality and Outcomes Evaluation and Measurements

What is the relationship between quality improvement and cost reduction?

Answer

High-quality care often reduces costs by preventing complications, avoiding unnecessary utilization, and delivering care efficiently. Reducing HACs, preventing readmissions, and eliminating waste all improve quality while reducing cost. Value-based payment models align these goals by rewarding both quality and efficiency.

#366 · Quality and Outcomes Evaluation and Measurements

What is a quality of life measure?

Answer

Quality of life measures capture the patient's overall experience of health and wellbeing across physical, emotional, social, and functional domains. Generic measures like the SF-36 or EQ-5D apply across conditions; disease-specific measures like the Minnesota Living with Heart Failure Questionnaire capture condition-specific impacts.

#367 · Quality and Outcomes Evaluation and Measurements

What is a hospital readmission reduction program?

Answer

CMS's Hospital Readmissions Reduction Program penalizes hospitals with excess 30-day readmissions for specific conditions including AMI, heart failure, pneumonia, COPD, hip/knee replacement, and CABG. Penalties are applied as a reduction in all Medicare payments, creating a strong financial incentive for effective discharge planning and case management.

#368 · Quality and Outcomes Evaluation and Measurements

What is the Agency for Healthcare Research and Quality (AHRQ)?

Answer

AHRQ is the lead federal agency for patient safety and healthcare quality research. It develops evidence-based tools and guidelines including TeamSTEPPS for teamwork training, the Comprehensive Unit-based Safety Program (CUSP), and the Patient Safety Indicators used in quality reporting.

#369 · Quality and Outcomes Evaluation and Measurements

What is TeamSTEPPS?

Answer

TeamSTEPPS (Team Strategies and Tools to Enhance Performance and Patient Safety) is an evidence-based teamwork training program developed by AHRQ and the Department of Defense. It teaches communication strategies including SBAR, closed-loop communication, and team huddles to improve patient safety in healthcare settings.

#370 · Quality and Outcomes Evaluation and Measurements

What is SBAR communication?

Answer

SBAR (Situation, Background, Assessment, Recommendation) is a structured communication tool for conveying critical information between healthcare providers. It provides a consistent format for clinical handoffs and escalation that reduces errors caused by incomplete or unstructured communication.

#371 · Quality and Outcomes Evaluation and Measurements

What is the purpose of public quality reporting?

Answer

Public quality reporting makes hospital and health plan quality data available to patients, employers, and policymakers so they can make informed choices about their care. CMS publicly reports hospital quality measures through the Hospital Compare website, creating reputational incentives for quality improvement alongside financial incentives.

#372 · Quality and Outcomes Evaluation and Measurements

What is the NCQA HEDIS measure for diabetes care?

Answer

HEDIS diabetes care measures include: A1c testing and control, blood pressure control, statin therapy use, nephropathy monitoring, and eye exam completion. Health plans are measured on the percentage of their diabetic population receiving these evidence-based care processes, which drives outreach and care management programs.

#373 · Quality and Outcomes Evaluation and Measurements

What is the HEDIS measure for mental health follow-up?

Answer

The HEDIS Follow-Up After Hospitalization for Mental Illness (FUH) measure tracks whether plan members have an outpatient mental health visit within 7 and 30 days after discharge from a psychiatric hospitalization. Poor follow-up is a major driver of psychiatric readmissions and is a priority quality target in behavioral health case management.

#374 · Quality and Outcomes Evaluation and Measurements

What is global quality in healthcare?

Answer

Global quality in healthcare encompasses the full range of IOM quality aims: safety, effectiveness, patient-centeredness, timeliness, efficiency, and equity. No single measure captures all dimensions; comprehensive quality programs track metrics across all six aims to identify trade-offs and gaps.

#375 · Quality and Outcomes Evaluation and Measurements

What is healthcare waste and how is it measured?

Answer

Healthcare waste includes low-value or unnecessary care, administrative inefficiency, fraud and abuse, and failures in care delivery and coordination. The National Academy of Medicine estimates that 25-30% of healthcare spending is waste. Case management reduces waste by preventing avoidable utilization and ensuring appropriate use of resources.

#376 · Rehabilitation Concepts and Strategies

What is the goal of rehabilitation?

Answer

Rehabilitation aims to restore a patient to the highest achievable level of function and independence in the least restrictive environment. It begins as early as medically feasible, addresses physical, cognitive, emotional, and vocational function, and always keeps the patient's own goals at the center of the plan.

#377 · Rehabilitation Concepts and Strategies

What is physical therapy (PT)?

Answer

Physical therapy focuses on mobility, strength, balance, and gross motor function. Physical therapists evaluate the patient's ability to move, transfer, walk, and manage stairs, develop exercise programs, train patients in assistive device use, and educate on home exercise programs to maintain gains after discharge.

#378 · Rehabilitation Concepts and Strategies

What is occupational therapy (OT)?

Answer

Occupational therapy focuses on a patient's ability to perform activities of daily living (ADLs) and instrumental activities of daily living (IADLs). OT also addresses cognitive function, fine motor skills, home safety, and adaptive equipment to support maximum independence in daily life.

#379 · Rehabilitation Concepts and Strategies

What is speech-language pathology (SLP)?

Answer

Speech-language pathology addresses communication disorders, swallowing difficulties (dysphagia), and cognitive-communication deficits. SLPs conduct modified barium swallow studies to determine safe food textures and evaluate aphasia, cognitive impairment, and voice disorders affecting communication.

#380 · Rehabilitation Concepts and Strategies

What are ADLs and IADLs?

Answer

ADLs (activities of daily living) are basic self-care tasks: bathing, dressing, grooming, toileting, transferring, continence, and eating. IADLs (instrumental activities of daily living) require higher function: medication management, meal preparation, finances, housekeeping, shopping, transportation, and telephone use.

#381 · Rehabilitation Concepts and Strategies

What is the Inpatient Rehabilitation Facility (IRF)?

Answer

An IRF is a specialty hospital or unit providing at least three hours of intensive rehabilitation therapy per day, requiring close physician supervision and an interdisciplinary team. Medicare requires patients to demonstrate they can tolerate and benefit from the intensive therapy. Common diagnoses include stroke, TBI, hip fracture, and spinal cord injury.

#382 · Rehabilitation Concepts and Strategies

What is the three-hour rule for IRF admission?

Answer

Medicare requires that IRF patients participate in at least three hours of intensive rehabilitation therapy per day, five days per week, or an equivalent 15 hours over seven days. A patient who can only tolerate 90 minutes per day does not meet this criterion and should be placed in a SNF or home health instead.

#383 · Rehabilitation Concepts and Strategies

What is a Skilled Nursing Facility (SNF)?

Answer

A SNF provides nursing care and rehabilitation therapy for patients who need daily skilled services but not acute hospital care. Therapy is at lower intensity than an IRF. Medicare covers SNF care after a qualifying three-day inpatient hospital stay (not observation status) with a specific daily co-insurance after day 20.

#384 · Rehabilitation Concepts and Strategies

What is Long-Term Acute Care (LTACH)?

Answer

LTACHs serve patients requiring prolonged acute-level care who are not ready for rehabilitation. Typical patients include those being weaned from mechanical ventilation, those with complex wounds, or those with multiple organ dysfunction. The average LTACH length of stay must exceed 25 days.

#385 · Rehabilitation Concepts and Strategies

What is home health rehabilitation?

Answer

Home health rehabilitation provides skilled therapy in the patient's home when the patient is homebound. Medicare covers home health when the patient has an active skilled need (PT, OT, or SLP) and is under a physician's plan of care. It is appropriate for patients who are medically stable but cannot easily travel to outpatient therapy.

#386 · Rehabilitation Concepts and Strategies

What is outpatient rehabilitation?

Answer

Outpatient rehabilitation provides therapy at a clinic or therapy center that the patient travels to. It is appropriate for patients who are ambulatory and functionally independent enough to travel but still need ongoing skilled therapy to improve or maintain function. It is the least resource-intensive rehabilitation setting.

#387 · Rehabilitation Concepts and Strategies

What is the Barthel Index?

Answer

The Barthel Index is a validated 10-item ADL scale widely used in stroke rehabilitation. It measures independence in feeding, bathing, grooming, dressing, bowel control, bladder control, toileting, transfers, ambulation, and stair climbing. Higher scores indicate greater independence.

#388 · Rehabilitation Concepts and Strategies

What is the Functional Independence Measure (FIM)?

Answer

The FIM is an 18-item scale measuring motor and cognitive function on a 7-point scale from total assistance to complete independence. It is used in IRF settings to document functional status on admission and discharge, justify the intensity of rehabilitation needed, and support Medicare payment under the IRF-PAI.

#389 · Rehabilitation Concepts and Strategies

What is the Katz Index of Independence in ADLs?

Answer

The Katz Index measures independence in six basic ADLs: bathing, dressing, toileting, transferring, continence, and feeding. It is widely used in assessment of older adults and predicts care needs and outcomes. Scores range from A (fully independent in all six) to G (dependent in all six).

#390 · Rehabilitation Concepts and Strategies

What is cognitive rehabilitation?

Answer

Cognitive rehabilitation is provided by neuropsychologists, occupational therapists, and speech-language pathologists for patients with memory problems, attention deficits, and executive function impairment following TBI, stroke, or dementia. It uses compensatory strategies, environmental modifications, and repetitive practice to restore or adapt cognitive function.

#391 · Rehabilitation Concepts and Strategies

What is vocational rehabilitation (VR)?

Answer

Vocational rehabilitation helps people with physical or mental disabilities prepare for, find, or maintain employment through evaluation, counseling, job skills training, assistive technology, and job placement. State VR agencies receive federal funding under the Rehabilitation Act of 1973 and are available to anyone with a disability that substantially impairs employment.

#392 · Rehabilitation Concepts and Strategies

What is supported employment?

Answer

Supported employment places individuals with significant disabilities directly in competitive integrated employment with ongoing job coaching and support (place-and-train model). It has the strongest evidence base for employment outcomes in people with serious mental illness, outperforming traditional train-and-place approaches.

#393 · Rehabilitation Concepts and Strategies

What is maximum medical improvement (MMI)?

Answer

MMI is the point at which a worker's condition has stabilized and further medical treatment is not expected to produce significant improvement. At MMI, permanent disability ratings are assigned and final return-to-work decisions are made. MMI does not mean full recovery; it means the condition has plateaued.

#394 · Rehabilitation Concepts and Strategies

What is a Functional Capacity Evaluation (FCE)?

Answer

An FCE is a comprehensive, objective assessment of a worker's physical abilities in relation to the demands of a specific job. It is typically performed by a PT or OT and provides data for return-to-work planning, establishing permanent work restrictions, and disability ratings in workers' compensation cases.

#395 · Rehabilitation Concepts and Strategies

What is a Job Demands Analysis (JDA)?

Answer

A JDA identifies the physical, cognitive, and environmental requirements of a specific job, including lifting requirements, postural demands, and cognitive complexity. The case manager compares JDA findings to the worker's FCE results to determine whether return to work is feasible and what accommodations may be needed.

#396 · Rehabilitation Concepts and Strategies

What is an impairment rating?

Answer

An impairment rating is a percentage assigned to represent permanent loss of function resulting from an injury, typically using the AMA Guides to the Evaluation of Permanent Impairment. It is used in workers' compensation to calculate permanent disability benefits and is assigned at MMI.

#397 · Rehabilitation Concepts and Strategies

What is the return-to-work (RTW) process?

Answer

RTW is the cornerstone of workers' compensation case management. It may be full duty, modified duty (original job with task or hour changes), or light duty (different tasks within current abilities). Early RTW prevents deconditioning, depression, and chronic disability while reducing compensation costs.

#398 · Rehabilitation Concepts and Strategies

What is the ADA (Americans with Disabilities Act) in rehabilitation?

Answer

The ADA requires employers with 15 or more employees to provide reasonable accommodations to qualified employees with disabilities. In rehabilitation, the case manager educates both the worker and employer about ADA rights, helps identify appropriate accommodations, and facilitates the interactive process between employer and employee.

#399 · Rehabilitation Concepts and Strategies

What is assistive technology (AT)?

Answer

Assistive technology refers to devices and equipment that help people with disabilities perform activities they would otherwise have difficulty completing. AT ranges from simple grab bars and reachers to complex powered wheelchairs with environmental control systems. Case managers arrange AT through assessment, prescription, and funding coordination.

#400 · Rehabilitation Concepts and Strategies

What is durable medical equipment (DME)?

Answer

DME is equipment that is medically necessary, durable (able to withstand repeated use), used for a medical purpose, not generally useful to healthy people, and appropriate for home use. Medicare Part B covers DME with a physician's order and certificate of medical necessity. The case manager arranges DME through in-network suppliers.

#401 · Rehabilitation Concepts and Strategies

What are home modifications in rehabilitation?

Answer

Home modifications improve safety and accessibility for patients with functional limitations. Examples include ramp installation, grab bars in bathrooms, stair lifts, widened doorways, and raised toilet seats. Funding sources include Medicaid HCBS waivers, Area Agencies on Aging, and nonprofit organizations like Rebuilding Together.

#402 · Rehabilitation Concepts and Strategies

What is stroke rehabilitation?

Answer

Stroke rehabilitation addresses hemiplegia, aphasia, dysphagia, cognitive impairment, depression, and fatigue through a coordinated team including PT, OT, SLP, and psychology. The intensity and setting of rehabilitation depend on the patient's ability to tolerate therapy; most patients begin in the acute hospital and progress through IRF, home health, or outpatient therapy.

#403 · Rehabilitation Concepts and Strategies

What is aphasia?

Answer

Aphasia is a language disorder from brain damage (most commonly stroke) that affects the ability to speak, understand, read, or write. It does not reflect a loss of intelligence. Speech-language pathology is the primary rehabilitation intervention. Case managers facilitate communication by allowing extra time and using visual supports.

#404 · Rehabilitation Concepts and Strategies

What is traumatic brain injury (TBI) rehabilitation?

Answer

TBI rehabilitation addresses physical, cognitive, behavioral, and vocational impairments through an interdisciplinary team including neuropsychology, OT, PT, SLP, and vocational rehabilitation. Long-term case management is needed for moderate-to-severe TBI as patients reintegrate into family, community, and employment roles.

#405 · Rehabilitation Concepts and Strategies

What is spinal cord injury (SCI) rehabilitation?

Answer

SCI rehabilitation addresses medical complications (pressure injuries, autonomic dysreflexia, neurogenic bowel and bladder), functional training, equipment prescription, home modifications, and community reintegration. SCI case management is lifelong due to the permanence of the disability and complexity of medical needs.

#406 · Rehabilitation Concepts and Strategies

What is autonomic dysreflexia?

Answer

Autonomic dysreflexia is a potentially life-threatening complication of SCI at T6 or above involving a sudden, dangerous spike in blood pressure triggered by a noxious stimulus below the level of injury (commonly a full bladder or bowel). Patient and caregiver education on recognition and emergency response is a priority in SCI case management.

#407 · Rehabilitation Concepts and Strategies

What is lymphedema management in cancer rehabilitation?

Answer

Lymphedema is a chronic complication of cancer treatment when axillary or pelvic lymph nodes are removed or irradiated, causing persistent swelling of the affected limb. Management by a certified lymphedema therapist includes complete decongestive therapy, compression garments, skin care, and patient education on self-management.

#408 · Rehabilitation Concepts and Strategies

What is cancer rehabilitation?

Answer

Cancer rehabilitation addresses functional consequences of cancer and its treatment including fatigue, weakness, lymphedema, neuropathy, cognitive effects of chemotherapy (chemo brain), and pain. It is appropriate throughout the cancer treatment trajectory and into survivorship, not only after treatment completion.

#409 · Rehabilitation Concepts and Strategies

What is pulmonary rehabilitation?

Answer

Pulmonary rehabilitation is a supervised exercise and education program for patients with chronic lung disease, particularly COPD. It improves exercise tolerance, reduces dyspnea, and decreases hospitalizations. Medicare Part B covers pulmonary rehabilitation for COPD with physician referral.

#410 · Rehabilitation Concepts and Strategies

What is cardiac rehabilitation?

Answer

Cardiac rehabilitation is a medically supervised program combining exercise training, education, and risk factor modification for patients after cardiac events including MI, bypass surgery, and heart failure. It reduces mortality, hospitalizations, and depression, and improves quality of life. Medicare Part B covers cardiac rehab for qualifying diagnoses.

#411 · Rehabilitation Concepts and Strategies

What is dysphagia?

Answer

Dysphagia is difficulty swallowing, common after stroke, head and neck surgery, or neurological conditions. Aspiration of food or liquid into the airway can cause aspiration pneumonia. SLPs assess dysphagia with modified barium swallow studies and recommend safe food textures and liquid consistencies.

#412 · Rehabilitation Concepts and Strategies

What is neurogenic bladder?

Answer

Neurogenic bladder results from damage to the nervous system controlling bladder function, common in SCI, MS, and Parkinson's disease. It causes urinary retention or incontinence requiring management with intermittent catheterization, medications, or other interventions coordinated by urology and case management.

#413 · Rehabilitation Concepts and Strategies

What is a prosthesis and prosthetic rehabilitation?

Answer

A prosthesis is an artificial limb or body part. Prosthetic rehabilitation following amputation involves fitting by a prosthetist, gait training with PT, ADL retraining with OT, and psychological support. The case manager coordinates the rehabilitation team, insurance authorization, and community reintegration.

#414 · Rehabilitation Concepts and Strategies

What is an orthotics?

Answer

An orthosis is a device applied externally to support, align, prevent, or correct a body part. Examples include ankle-foot orthoses (AFOs) for foot drop, back braces for spinal conditions, and knee orthoses after ligament injury. Orthotists design and fit these devices; case managers coordinate prescription and funding.

#415 · Rehabilitation Concepts and Strategies

What is the role of case management in managing workers' compensation?

Answer

In workers' compensation case management, the case manager coordinates medical treatment, facilitates communication between the injured worker, employer, and insurer, monitors recovery progress, coordinates FCE and JDA, and focuses on appropriate and timely return to work at the earliest safe opportunity.

#416 · Rehabilitation Concepts and Strategies

What is disability management?

Answer

Disability management is a workplace program designed to minimize the impact of injury or illness on a worker's ability to participate in the workforce. It includes early intervention, modified duty programs, rehabilitation coordination, and return-to-work support to reduce lost work time and disability costs.

#417 · Rehabilitation Concepts and Strategies

What is early intervention in pediatric rehabilitation?

Answer

Early intervention programs provide services for children from birth to age three with developmental delays or disabilities. Services include PT, OT, SLP, and developmental therapy, delivered in natural environments such as the home. Case managers connect families with early intervention services as soon as developmental concerns are identified.

#418 · Rehabilitation Concepts and Strategies

What is the transition from pediatric to adult care?

Answer

Transition from pediatric to adult healthcare is a planned, purposeful process for adolescents and young adults with chronic conditions or disabilities. It prepares the patient for adult models of care where family is less central, helps them develop self-advocacy skills, and establishes new adult care providers before the transition occurs.

#419 · Rehabilitation Concepts and Strategies

What is a rehabilitation program for patients with chronic pain?

Answer

Chronic pain rehabilitation uses an interdisciplinary team to address the physical, psychological, and functional dimensions of persistent pain. Components include physical reconditioning, CBT, pain education, relaxation techniques, and occupational therapy for functional restoration, aiming to improve function and quality of life regardless of pain level.

#420 · Rehabilitation Concepts and Strategies

What is the IRF-PAI (Inpatient Rehabilitation Facility Patient Assessment Instrument)?

Answer

The IRF-PAI is the required CMS assessment instrument for all IRF admissions and discharges. It captures functional status using FIM scores, medical complexity, rehabilitation goals, and predicted length of stay, and is used to calculate the prospective payment amount for each IRF stay.

#421 · Rehabilitation Concepts and Strategies

What is the role of neuropsychology in rehabilitation?

Answer

Neuropsychologists conduct comprehensive evaluations of cognitive function after brain injury or neurological illness, identifying strengths and deficits that guide rehabilitation planning. They also provide therapy for emotional adjustment to disability and cognitive rehabilitation, and consult on decision-making capacity.

#422 · Rehabilitation Concepts and Strategies

What is community reintegration in rehabilitation?

Answer

Community reintegration is the process of supporting a patient's return to meaningful roles in the community after illness or disability, including family roles, social participation, employment, and leisure activities. Case managers address transportation, access barriers, peer support, and vocational rehabilitation as part of reintegration planning.

#423 · Rehabilitation Concepts and Strategies

What is palliative rehabilitation?

Answer

Palliative rehabilitation provides rehabilitation services to patients with serious or life-limiting illness with the goal of maximizing quality of life and function rather than achieving recovery. It may include positioning, range-of-motion exercises, energy conservation education, and adaptive equipment to maintain comfort and participation.

#424 · Rehabilitation Concepts and Strategies

What is chronic pain disability and its management?

Answer

Chronic pain disability occurs when pain significantly limits function and work capacity over time. Management integrates medical pain management, functional restoration through interdisciplinary rehabilitation, psychological treatment, occupational modifications, and vocational counseling rather than relying solely on medication or surgery.

#425 · Rehabilitation Concepts and Strategies

What is fall prevention in the rehabilitation setting?

Answer

Fall prevention in rehabilitation settings includes environmental safety assessment, supervised therapy with progressive challenge, patient and caregiver education on fall risk and home safety, balance training, medication review, and assistive device training. Falls in rehabilitation settings can set back recovery and cause additional injury.

#426 · Rehabilitation Concepts and Strategies

What is the role of case management in amputee rehabilitation?

Answer

Amputee case management coordinates surgical wound care, prosthetic fitting and training, PT for gait training, OT for ADL adaptation, psychological support for body image and adjustment, and vocational rehabilitation for return to work. The multidisciplinary amputation team typically includes surgery, physiatry, prosthetics, PT, OT, and social work.

#427 · Rehabilitation Concepts and Strategies

What is physiatry (physical medicine and rehabilitation)?

Answer

Physiatry is the medical specialty focused on diagnosing and treating musculoskeletal and neurological conditions that affect function. Physiatrists lead inpatient and outpatient rehabilitation teams, prescribe therapy and adaptive equipment, manage pain and spasticity, and coordinate the rehabilitation care plan in complex cases.

#428 · Rehabilitation Concepts and Strategies

What is deconditioning and why does it matter?

Answer

Deconditioning is the deterioration of physical function from inactivity, common in hospitalized patients. Even brief periods of bed rest cause muscle weakness, cardiorespiratory deconditioning, and functional decline. Early mobilization during hospitalization prevents deconditioning and reduces complications, length of stay, and discharge to higher-care settings.

#429 · Rehabilitation Concepts and Strategies

What is the role of the case manager in post-acute care planning?

Answer

The case manager assesses the patient's rehabilitation needs and potential, discusses post-acute options with the patient and family, communicates clinical needs to the receiving facility, coordinates the transfer with complete documentation, and monitors the patient's progress and goals in the post-acute setting.

#430 · Rehabilitation Concepts and Strategies

What is wheelchair evaluation and prescription?

Answer

Wheelchair evaluation determines the appropriate type (manual versus power, standard versus tilt/recline), size, seating system, and accessories for a patient's specific functional needs and environment. A certified rehabilitation technology supplier or seating specialist conducts the evaluation; the physician prescribes; Medicare Part B funds the chair with a certificate of medical necessity.

#431 · Rehabilitation Concepts and Strategies

What is the role of case management in multiple sclerosis (MS)?

Answer

MS case management coordinates disease-modifying therapy, symptom management, rehabilitation across PT, OT, and SLP, fatigue management, emotional support, and ADA accommodations for employment. MS is characterized by relapsing and progressive courses requiring flexible care planning that adjusts to changing functional status.

#432 · Rehabilitation Concepts and Strategies

What is the role of case management in Parkinson's disease rehabilitation?

Answer

Parkinson's disease rehabilitation addresses gait freezing, balance impairment, fall risk, speech changes, swallowing difficulties, and cognitive impairment through coordinated PT, OT, and SLP services. Evidence-based programs such as LSVT BIG (PT) and LSVT LOUD (SLP) are specifically designed for Parkinson's-related motor and speech changes.

#433 · Rehabilitation Concepts and Strategies

What is the Rehabilitation Act of 1973?

Answer

The Rehabilitation Act of 1973 provides the legal basis for federal vocational rehabilitation programs serving people with disabilities, prohibits disability discrimination by federal agencies and federally funded programs, and establishes protections later expanded by the ADA. State VR agencies operate under this Act.

#434 · Rehabilitation Concepts and Strategies

What is the role of case management in traumatic amputation?

Answer

Following traumatic amputation, case management coordinates emergency and surgical care, psychological support for acute grief and body image adjustment, prosthetic evaluation and fitting, vocational rehabilitation, and long-term community integration. Phantom limb pain is a common complication requiring multidisciplinary pain management.

#435 · Rehabilitation Concepts and Strategies

What is a rehabilitation potential assessment?

Answer

Rehabilitation potential assessment evaluates whether a patient is likely to improve with rehabilitation services, considering cognitive ability to learn and carry over new skills, motivation, medical stability, social support, and the expected trajectory of the underlying condition. It guides level-of-care decisions and goal-setting in the rehabilitation care plan.

#436 · Ethical Legal And Practice Standards

What are the four bioethical principles?

Answer

The four bioethical principles from Beauchamp and Childress are autonomy (patient's right to make their own decisions), beneficence (acting in the patient's best interests), non-maleficence (avoiding harm), and justice (fair and equitable treatment). These principles sometimes conflict, requiring ethical reasoning to navigate.

#437 · Ethical Legal And Practice Standards

What is veracity in healthcare ethics?

Answer

Veracity is the ethical obligation to be truthful with patients, families, payers, and employers. It requires honest documentation, honest communication about coverage limitations, and honest disclosure when mistakes are made. Case managers who falsify documentation violate veracity and expose themselves to legal liability.

#438 · Ethical Legal And Practice Standards

What is fidelity in healthcare ethics?

Answer

Fidelity means being faithful to commitments, following through on promised referrals, maintaining confidentiality, and honoring the professional relationship. A case manager who fails to follow through on arranged services after promising the patient has violated the principle of fidelity.

#439 · Ethical Legal And Practice Standards

What is patient autonomy?

Answer

Autonomy is the patient's right to make their own healthcare decisions. Respecting autonomy means providing full and truthful information, ensuring the patient understands it, and honoring their decisions even when you disagree. Competent adults have the right to refuse any treatment, including life-saving treatment.

#440 · Ethical Legal And Practice Standards

What is informed consent?

Answer

Informed consent requires three conditions: decision-making capacity (ability to understand and reason), adequate information (diagnosis, treatment, risks, benefits, alternatives, and consequences of refusal), and voluntariness (no coercion or undue influence). The absence of any one element invalidates consent.

#441 · Ethical Legal And Practice Standards

What is decision-making capacity?

Answer

Decision-making capacity is a clinical determination assessing whether a patient can understand relevant information, appreciate how it applies to their situation, reason about the options, and communicate a consistent choice. It is not the same as legal competence, which is determined by a court. Capacity may vary across decisions and over time.

#442 · Ethical Legal And Practice Standards

What is the substituted judgment standard?

Answer

The substituted judgment standard directs a surrogate decision-maker to make the decision the patient would have made if capable, based on the patient's previously expressed values and wishes. It is applied when the patient lacks capacity but their preferences are known. The surrogate sets aside their own preferences.

#443 · Ethical Legal And Practice Standards

What is the best interests standard?

Answer

The best interests standard applies when a patient lacks decision-making capacity and their preferences are unknown. The surrogate makes a decision based on what a reasonable person in the patient's circumstances would want. It is a fallback standard when substituted judgment cannot be applied.

#444 · Ethical Legal And Practice Standards

What is a healthcare proxy (durable power of attorney for healthcare)?

Answer

A healthcare proxy is a legal document designating a specific person to make healthcare decisions when the patient lacks capacity. Because the proxy can respond to unanticipated situations, it is more flexible than a living will. It is the most commonly recommended advance directive for most patients.

#445 · Ethical Legal And Practice Standards

What is a living will?

Answer

A living will is a patient-authored legal document specifying treatment preferences for situations in which the patient cannot make decisions, such as terminal illness or permanent unconsciousness. Unlike a proxy designation, it speaks only to anticipated scenarios and cannot be adapted to unforeseen circumstances.

#446 · Ethical Legal And Practice Standards

What is POLST?

Answer

POLST (Physician Orders for Life-Sustaining Treatment) is a medical order signed by a physician that translates a patient's goals into immediately actionable treatment orders covering resuscitation, hospitalization preferences, and artificial nutrition. It travels with the patient across care settings and can be acted on by emergency responders.

#447 · Ethical Legal And Practice Standards

What is a DNR/DNI order?

Answer

A DNR (Do Not Resuscitate) order specifies that the patient should not receive CPR if the heart stops. A DNI (Do Not Intubate) order specifies no mechanical ventilation. These are physician orders specific to individual patients and must be explicitly re-ordered when a patient transfers between settings.

#448 · Ethical Legal And Practice Standards

What is Medical Aid in Dying (MAID)?

Answer

MAID is legal in a growing number of states and allows terminally ill patients with decision-making capacity to request a prescription for medication to end their life. Patients must self-administer the medication. Case managers in states where MAID is legal must be familiar with requirements and their organization's policies.

#449 · Ethical Legal And Practice Standards

What is HIPAA?

Answer

HIPAA (Health Insurance Portability and Accountability Act) is the federal law governing the use and disclosure of protected health information (PHI). The Privacy Rule sets standards for permitted uses and disclosures. The Security Rule protects electronic PHI. Violations carry civil and criminal penalties.

#450 · Ethical Legal And Practice Standards

What is PHI under HIPAA?

Answer

PHI (Protected Health Information) is any individually identifiable health information held by a covered entity or business associate. It includes any information that could identify the patient (name, date of birth, SSN, address, dates of service) combined with health information. PHI must be protected in all forms: paper, electronic, and verbal.

#451 · Ethical Legal And Practice Standards

What are HIPAA permitted disclosures without authorization?

Answer

HIPAA permits disclosure without patient authorization for treatment (sharing with the care team), payment (billing purposes), and healthcare operations (quality review, training, auditing). These three purposes cover most routine case management disclosures. All other disclosures generally require written patient authorization.

#452 · Ethical Legal And Practice Standards

What is the minimum necessary standard under HIPAA?

Answer

The minimum necessary standard requires limiting PHI disclosures to the minimum amount needed to accomplish the intended purpose. A case manager should not share a patient's complete record when only a medication list is needed. This standard applies to all disclosures except for treatment purposes within the care team.

#453 · Ethical Legal And Practice Standards

What is the HIPAA Security Rule?

Answer

The HIPAA Security Rule requires covered entities to implement administrative, physical, and technical safeguards to protect electronic PHI. This includes encryption, access controls, audit logs, and workforce training. Electronic PHI in case management systems must be protected against unauthorized access and breach.

#454 · Ethical Legal And Practice Standards

What is confidentiality in case management?

Answer

Confidentiality is the ethical and legal obligation to protect patients' private health information from unauthorized disclosure. Case managers must safeguard PHI in all forms, use minimum necessary information, obtain proper authorization for non-routine disclosures, and understand exceptions such as mandatory reporting and duty to warn.

#455 · Ethical Legal And Practice Standards

What are mandatory reporting obligations?

Answer

Mandatory reporting laws require case managers to report certain situations to authorities regardless of confidentiality. These include suspected child abuse or neglect, suspected elder abuse, and in some states certain communicable diseases and credible threats of violence against identified third parties. The reporting obligation exists even without confirmed evidence; reasonable suspicion is sufficient.

#456 · Ethical Legal And Practice Standards

What is the duty to warn (Tarasoff obligation)?

Answer

The duty to warn, based on the Tarasoff case, requires mental health professionals and in some states case managers to breach confidentiality when a patient poses a serious and credible threat of violence against an identified third party. The obligation is to warn the potential victim and notify law enforcement.

#457 · Ethical Legal And Practice Standards

What is negligence in case management?

Answer

Negligence requires four elements: duty (the case manager had an obligation to the patient), breach (the standard of care was not met), causation (the breach caused harm), and damages (the patient suffered harm). Case managers can be liable for negligence through failures in assessment, communication, referral, and follow-up.

#458 · Ethical Legal And Practice Standards

What is malpractice?

Answer

Malpractice is professional negligence. A case manager can be held liable for malpractice if their failure to assess adequately, communicate important information, make appropriate referrals, or follow through on coordination commitments resulted in patient harm. Documentation is the primary evidence in malpractice cases.

#459 · Ethical Legal And Practice Standards

What is abandonment in case management?

Answer

Case management abandonment occurs when a case manager terminates a professional relationship without adequate notice and without providing the patient time to establish alternative care. Proper case closure includes patient communication, documentation, and referral to alternative resources. A patient voluntarily disenrolling is not abandonment.

#460 · Ethical Legal And Practice Standards

What is conflict of interest in case management?

Answer

Conflict of interest occurs when the case manager's personal or professional interests interfere with their obligation to act in the patient's best interests. Case managers employed by insurers must identify and manage the tension between employer financial interests and patient needs through ethical practice and transparent communication.

#461 · Ethical Legal And Practice Standards

What is scope of practice in case management?

Answer

Scope of practice defines the range of professional activities a case manager is authorized to perform based on their license, certification, employer policies, and applicable law. Tasks outside one's scope must be referred to the appropriate professional. Scope is not determined by experience or confidence alone.

#462 · Ethical Legal And Practice Standards

What is the CCMC Code of Professional Conduct?

Answer

The CCMC Code requires CCM certificants to practice with integrity, maintain competence, avoid and disclose conflicts of interest, protect confidentiality, and promote the interests of clients above personal or organizational financial interests. Violations can result in suspension or revocation of the CCM credential.

#463 · Ethical Legal And Practice Standards

What is the CCM (Certified Case Manager) credential?

Answer

The CCM credential is awarded by the Commission for Case Manager Certification to candidates who hold an active license or certification in a health or human services profession and have completed supervised case management practice. It is the primary professional certification for case managers across all settings.

#464 · Ethical Legal And Practice Standards

What are the CCM exam eligibility requirements?

Answer

CCM candidates must hold an active, acceptable license or certification in a health or human services discipline and have completed a defined amount of supervised case management work experience. Both an appropriate credential and supervised experience are required; work experience alone does not satisfy the requirements.

#465 · Ethical Legal And Practice Standards

What is the CCMC and what does it do?

Answer

The Commission for Case Manager Certification (CCMC) awards and administers the CCM credential, sets eligibility and examination standards, maintains the Code of Professional Conduct, and requires continuing education for credential renewal. It is distinct from CMSA, the professional association.

#466 · Ethical Legal And Practice Standards

What is continuing education for CCM renewal?

Answer

CCM holders must obtain a specified number of continuing education hours in approved content areas or pass the CCM examination again during each renewal cycle. The obligation is the credential holder's personal responsibility. The specific requirements are set by the CCMC and updated periodically.

#467 · Ethical Legal And Practice Standards

What are professional boundaries in case management?

Answer

Professional boundaries define the appropriate limits of the therapeutic relationship. Commonly crossed boundaries include giving personal advice outside the professional role, accepting gifts of significant value, sharing inappropriate personal information, and developing social or romantic relationships with patients. Violations damage trust and create legal risk.

#468 · Ethical Legal And Practice Standards

What is patient advocacy?

Answer

Patient advocacy is the active promotion and defense of a patient's rights, needs, and preferences within the healthcare system. It includes ensuring patients have information to make informed decisions, ensuring their voice is heard in care decisions, and challenging denials, discriminatory practices, or unsafe care on the patient's behalf.

#469 · Ethical Legal And Practice Standards

What are patient rights?

Answer

Patient rights include the right to receive care without discrimination, privacy and confidentiality, dignified treatment, complete and accurate information about diagnosis and treatment, participation in treatment decisions, refusal of treatment, access to medical records, the right to file a grievance, and access to a qualified interpreter.

#470 · Ethical Legal And Practice Standards

What is the Americans with Disabilities Act (ADA)?

Answer

The ADA prohibits discrimination against people with disabilities in employment, public accommodations, transportation, and government services. In healthcare, the ADA requires accessible facilities and services. In employment, it requires reasonable accommodations for qualified employees with disabilities from employers with 15 or more employees.

#471 · Ethical Legal And Practice Standards

What is the ethical decision-making framework in case management?

Answer

Ethical decision-making follows a structured process: identify the ethical issue and stakeholders, gather information, identify options and likely consequences, apply ethical principles, consult when needed, decide and document reasoning, and evaluate outcomes afterward. The process provides a defensible approach to complex dilemmas.

#472 · Ethical Legal And Practice Standards

What is an ethics committee?

Answer

An ethics committee is a multidisciplinary group within a healthcare organization that provides consultation on difficult ethical questions, reviews policies with ethical implications, and provides education on bioethical issues. Case managers request ethics consultations when clinical and family disagreements about care cannot be resolved through other means.

#473 · Ethical Legal And Practice Standards

What is justice in bioethics?

Answer

Justice in bioethics refers to fair and equitable treatment of individuals and the fair distribution of healthcare resources. It requires the case manager to recognize and challenge disparities that result in worse care for patients from marginalized groups and to advocate for equitable access to needed services.

#474 · Ethical Legal And Practice Standards

What is the difference between capacity and competence?

Answer

Capacity is a clinical determination made by a qualified clinician assessing whether the patient can understand, appreciate, reason, and communicate about a specific healthcare decision. Competence is a legal determination made by a court. A patient can have capacity for some decisions but not others, and capacity can change over time.

#475 · Ethical Legal And Practice Standards

What is the surrogate decision-maker hierarchy?

Answer

When a patient lacks capacity and has no designated proxy, the hierarchy for surrogate decision-making is generally: spouse or domestic partner, then adult children, then parents, then adult siblings, then other relatives. State law governs this hierarchy and it varies by state. The surrogate applies the substituted judgment or best interests standard.

#476 · Ethical Legal And Practice Standards

What is a Hospital-Issued Notice of Non-Coverage (HINN)?

Answer

A HINN informs a Medicare beneficiary that their hospital stay is no longer covered because they no longer meet inpatient medical necessity criteria. It triggers the patient's right to a free, expedited review by the QIO (Quality Improvement Organization) without losing the right to appeal if the review upholds the non-coverage decision.

#477 · Ethical Legal And Practice Standards

What is a grievance in health insurance?

Answer

A grievance is a complaint about the quality of care or service provided by a health plan or its contracted providers, as distinct from an appeal, which challenges a coverage or payment decision. Health plans are required to have grievance procedures, and CMS sets specific response time requirements for Medicare Advantage grievances.

#478 · Ethical Legal And Practice Standards

What is fraud versus abuse in healthcare billing?

Answer

Healthcare fraud involves intentional deception to obtain unauthorized payment, such as billing for services not provided. Abuse refers to practices that are inconsistent with sound medical, business, or fiscal practices resulting in unnecessary cost, such as billing for services not medically necessary. Fraud is intentional; abuse may be unintentional.

#479 · Ethical Legal And Practice Standards

What is the False Claims Act?

Answer

The False Claims Act is a federal law that imposes liability on individuals and entities that submit false claims for payment to the federal government, including Medicare and Medicaid. It includes whistleblower protections and allows private citizens to file lawsuits on behalf of the government (qui tam actions) and share in recovered funds.

#480 · Ethical Legal And Practice Standards

What are Stark Laws (physician self-referral laws)?

Answer

The Stark Laws prohibit physicians from referring Medicare patients to healthcare entities with which the physician (or immediate family member) has a financial relationship, unless an exception applies. They are designed to prevent conflicts of interest in referral decisions that could increase costs or compromise patient care.

#481 · Ethical Legal And Practice Standards

What is the role of the case manager in recognizing ethical issues?

Answer

The case manager is often the team member with the most comprehensive view of the patient's situation and is positioned to identify ethical issues that may not be visible to individual clinicians. Recognizing when patient autonomy is being overridden, when conflicts of interest exist, or when care disparities are occurring is a core case management competency.

#482 · Ethical Legal And Practice Standards

What is the principle of non-maleficence in case management practice?

Answer

Non-maleficence, or 'first, do no harm,' requires the case manager to consider whether the potential harms of an intervention outweigh its benefits, to avoid actions that create unnecessary risk, and to speak up when a care plan element poses unacceptable risk to a patient without proportionate benefit.

#483 · Ethical Legal And Practice Standards

What is the principle of beneficence in case management?

Answer

Beneficence means actively working in the patient's best interests. In case management, it requires understanding what 'good' means from the patient's own perspective rather than imposing a clinically defined standard. Beneficence does not override autonomy; it operates alongside it, informing advocacy and care planning.

#484 · Ethical Legal And Practice Standards

What is the ethical issue of resource allocation?

Answer

Resource allocation involves decisions about how to distribute limited healthcare resources equitably across a patient population. Case managers face resource allocation decisions in utilization management, in prioritizing caseload attention, and in advocating for services. Justice principles guide allocation decisions toward equity and proportional need.

#485 · Ethical Legal And Practice Standards

What is the role of case management in advance directive completion?

Answer

Case managers educate patients about advance directives, facilitate discussions about values and preferences, ensure completed documents are placed in the medical record and travel with the patient across settings, and communicate advance directives to all members of the care team to ensure the patient's wishes are honored.

#486 · Ethical Legal And Practice Standards

What is the Patient Self-Determination Act (PSDA)?

Answer

The PSDA requires all hospitals, nursing homes, home health agencies, hospices, and managed care organizations that receive Medicare and Medicaid funding to inform patients of their rights to make healthcare decisions and to create advance directives. It does not require patients to have an advance directive, only that they be informed of the right.

#487 · Ethical Legal And Practice Standards

What is the role of case management in protecting vulnerable adults?

Answer

Case managers are often the first to identify signs of elder abuse, self-neglect, or exploitation in vulnerable adults. They are mandatory reporters in most states, meaning they must report reasonable suspicion to Adult Protective Services regardless of patient preference or family explanation, and must document their observations and reporting actions.

#488 · Ethical Legal And Practice Standards

What is the role of cultural competence in ethical practice?

Answer

Cultural competence (and the more current concept of cultural humility) is an ethical obligation because failure to understand and respect cultural differences can result in harmful misunderstandings, ineffective care, and violations of patient autonomy. Ethical case management requires adapting communication and care planning to the patient's cultural context.

#489 · Ethical Legal And Practice Standards

What is the difference between ethics and law in case management practice?

Answer

Ethics represents what is morally right and is governed by professional codes and principles. Law represents what is legally required and is enforced through courts and regulatory agencies. Sometimes law and ethics align; sometimes they diverge. Case managers must meet both legal standards and ethical obligations, and when they conflict, seek consultation.

#490 · Ethical Legal And Practice Standards

What is the CMSA Code of Ethics?

Answer

The CMSA Code of Ethics outlines case managers' ethical obligations to their clients, colleagues, employers, and the broader society. Key principles include respecting client autonomy, maintaining confidentiality, practicing within scope, pursuing continuing education, and advocating for vulnerable populations. It complements the CCMC Code of Professional Conduct.

#491 · Ethical Legal And Practice Standards

What is the role of the ombudsman in healthcare?

Answer

An ombudsman is an independent advocate who investigates and resolves complaints from patients about the quality of care, treatment, or services from a healthcare organization or health plan. Long-term care ombudsmen are federally mandated to serve nursing home residents. Case managers refer patients to ombudsmen when internal complaint processes fail.

#492 · Ethical Legal And Practice Standards

What is the role of the case manager in protecting patient privacy during telehealth?

Answer

During telehealth encounters, case managers must ensure the communication platform is HIPAA-compliant, the patient is in a private location, consent to telehealth is documented, and PHI transmitted electronically is encrypted. Consumer video platforms (Zoom, FaceTime) without a BAA are generally not compliant for clinical use.

#493 · Ethical Legal And Practice Standards

What is a business associate agreement (BAA)?

Answer

A BAA is a required HIPAA contract between a covered entity and a vendor who handles PHI on its behalf (a business associate). Examples include claims clearinghouses, IT vendors hosting EHRs, and telehealth platforms. Case managers must ensure their employers have BAAs in place with all vendors who access PHI.

#494 · Ethical Legal And Practice Standards

What is the role of case management in addressing health disparities ethically?

Answer

The justice principle obligates case managers to identify when patients are receiving inferior care based on race, income, disability, or other factors, and to advocate for equitable care. This may include requesting clinical review of decisions that appear to reflect bias and connecting underserved patients with advocacy organizations.

#495 · Ethical Legal And Practice Standards

What is the role of case management in supporting patients who refuse recommended treatment?

Answer

A competent patient has the absolute right to refuse any treatment. The case manager respects this right, ensures the patient is fully informed of the risks and alternatives, documents the conversation thoroughly, explores the reasons for refusal to address any misunderstandings, and maintains the therapeutic relationship rather than withdrawing care.

#496 · Ethical Legal And Practice Standards

What is the role of case management documentation in risk management?

Answer

Complete, accurate, and timely documentation is the primary evidence in any legal challenge to case management practice. Documentation must reflect the assessment findings, care plan rationale, patient communications, follow-up actions, and clinical reasoning. What is not documented is presumed not to have occurred in a legal context.

#497 · Ethical Legal And Practice Standards

What is the role of case management in supporting culturally diverse end-of-life decision-making?

Answer

End-of-life practices and beliefs vary significantly across cultures, including beliefs about disclosure of terminal prognosis, the role of family versus individual in decision-making, and the acceptability of withdrawing life support. Case managers explore these cultural dimensions without imposing a Western individualistic framework on the patient's decision-making process.

#498 · Ethical Legal And Practice Standards

What is professional self-care and its relevance to case management ethics?

Answer

Case managers have an ethical obligation to maintain their own competence and wellbeing, because burnout, compassion fatigue, and impaired judgment can harm patients. Self-care, supervision, and seeking support are professional ethical responsibilities, not optional self-indulgences, particularly in case management roles with high emotional exposure.

#499 · Ethical Legal And Practice Standards

What is the CCM exam content domain for ethical, legal, and practice standards?

Answer

The ethical, legal, and practice standards domain of the CCM exam covers HIPAA and confidentiality, scope of practice, professional conduct, advance directives, mandatory reporting, patient rights, legal concepts (negligence, malpractice, abandonment), conflict of interest, and the CCMC Code of Professional Conduct.

#500 · Ethical Legal And Practice Standards

What is the fundamental purpose of case management?

Answer

The fundamental purpose of case management is to meet the individual needs of the patient and their support system while promoting quality and cost-effective outcomes. It balances the patient's clinical and psychosocial needs with available resources, always placing the patient's best interests above organizational financial interests.